

**GUIDE**

Vytalix Investor & Execution Pack — Start Here

Vytalix company-build documentation · reference START_HERE

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

DATE

8 September 2026

VERSION

1.0 · Confidential draft

Built 8 September 2026 from the Vytalix company-build repository. Everything is a draft for a pre-formation company: nothing here claims traction, approval, partners or revenue that do not exist. Read `FACTS_BASE.md` first.

Read in this order

1. `docs/23_INVESTOR_READINESS_AND_WINNING_CONCEPT` — what to do to become investor-ready, the Maternal Continuity Layer concept, and the government-contract playbook.
2. `docs/00_MASTER_AUDIT` — where the business stands (score 11/100) and the top problems and opportunities.
3. `docs/19_FIRST_30_DAYS` and `docs/18_90_DAY_EXECUTION_PLAN` — day-by-day and 90-day plans from Monday 14 September 2026.
4. `assets/INVESTOR_ONE_PAGER` and `exports/pptx/INVESTOR_PITCH_DECK.pptx` — what investors see (only after the pre-conditions in doc 23 are met).
5. `docs/21_MATERNALINK_PRICING` — pricing per product and annual service.
6. `investors/` — 316-investor CRM (xlsx), Top 25, call lists, outreach scripts, dashboard.

What is in the pack

Folder	Contents
<code>docs/</code>	24 strategy, product, finance, legal and operations documents (Markdown)
<code>exports/docx/</code>	The same documents as Word files
<code>exports/pptx/</code>	Investor pitch deck, corporate presentation, sales deck, partnership deck (PowerPoint, with speaker notes)
<code>assets/</code>	Company profile, service catalogue, pricing structure, proposal template, business plan, one-pager, content calendar, launch campaign
<code>finance/</code>	3-year financial model workbook (three scenarios) and its builder script
<code>investors/</code>	Investor CRM (xlsx and csv), Top 25, call lists, outreach engine, pipeline dictionary, non-dilutive routes
<code>product/</code>	MaternaLink PRD, MVP spec, roadmap, feature addendum
<code>website/</code>	Vytalix corporate website (14 pages, static)
<code>maternalink-site/</code>	MaternaLink product website with demo dashboard (8 pages, static)
<code>operations/</code>	KPI dashboard and pipeline tracker templates
<code>brand/</code>	Vytalix logo files

Before anything goes to an investor or a government

Entity incorporated · IP deed signed · 2025 clinical claims retired · prototype behind authentication with synthetic data · one-pager and deck updated with real dates and names · data room assembled (index in doc 23 §2).



STRATEGY & OPERATIONS

Vytalix Master Audit

Vytalix company-build documentation · reference 00_MASTER_AUDIT

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Contents

EXECUTIVE VIEW	1
1. WHAT WAS REVIEWED	2
2. AREA-BY-AREA SCORES (0–10)	3
3. CURRENT BUSINESS SCORE: 11 / 100	4
4. SWOT	5
5. TOP 10 PROBLEMS (ranked by damage × urgency)	6
6. TOP 10 OPPORTUNITIES (ranked by value × feasibility)	7
7. WHAT CREATES REVENUE, CREDIBILITY, INVESTOR CONFIDENCE, AND RISK	8
8. WHAT TO DELETE, REDESIGN, AUTOMATE, PRIORITISE	9
9. IMMEDIATE ACTIONS (next 7 days)	10
PRIORITIES · RISKS · NEXT ACTIONS	11

Audit date: 7 September 2026 · **Auditor:** Founding Executive Team (AI-assisted) · **Status labels used:** VERIFIED · KNOWN · ASSUMPTION · ESTIMATE · TO VALIDATE · PROPOSED · NOT PUBLICLY AVAILABLE

Source of truth for facts: /FACTS_BASE.md

EXECUTIVE VIEW

- Vytalix does not yet exist as a business.** There is a name (spelled two ways), a brief, an empty code repository, one concept deck, one unsolicited government proposal and one prototype dashboard whose ownership is undocumented. Everything else must be built.
- Current business score: 11 / 100.** This is a pre-formation score, not a judgement on the idea. The idea is credible; the execution infrastructure is absent.
- The single most dangerous issue is IP and ownership.** The MaternaLink material is signed by David Agunede as "Founder & CEO, Maternal Link". Until there is a written agreement, Vytalix cannot truthfully tell an investor, a hospital or a ministry that it owns, licenses or controls MaternaLink.
- The second most dangerous issue is claims.** The 2025 deck promises AI that "predicts pre-eclampsia, sepsis, haemorrhage, thromboembolism" and reduces maternal mortality, with no data, no model, no ethics approval and no regulatory pathway. Used externally, this exposes Vytalix to regulatory, reputational and investor-diligence failure. It is also entirely fixable by re-scoping.
- The fastest path to a real company is not MaternaLink.** It is a founder-led advisory/consulting practice that generates cash and credibility within 60–90 days, while MaternaLink is re-based, protected and validated. The 300-investor CRM built alongside this audit is for the equity story, which becomes fundable once the problems below are closed.

1. WHAT WAS REVIEWED

#	Asset	Where	What it is	Verdict
1	GitHub repo elkelvinwilliams/Vitalyx	GitHub	One file (README.md , "# Vitalyx"). No code, docs, site or brand.	EMPTY (VERIFIED)
2	Founder brief ("Master Business-Build Prompt")	This session	A high-quality, complete specification of the company to be built. The best asset Vytalix currently has.	STRONG (KNOWN)
3	"Black Maternal health" deck (Google Slides + PPTX, Oct–Nov 2025, 13 slides)	Google Drive	MATERNA-LINK multimodal AI concept: clinical + voice + social data → "Maternal Instability Score" → clinician alerts. Cites UK Black maternal mortality disparity. Lists a tech stack and an "expand to NHS trusts" ambition.	CONCEPT ONLY. No data, model, validation, partner, ethics, regulatory route. Over-claims. (VERIFIED)

#	Asset	Where	What it is	Verdict
4	Ekiti State Ministry of Health proposal + follow-up email (18 Mar 2026, David Agunede, "Founder & CEO, Maternal Link")	Gmail	Phased "maternal monitoring programme": 15,000 women £4.5m → 30,000 £9m → 120,000 £36m; £300/woman/year. Claims 99.5–99.9% uptime target, offline-first, cloud backup. Attachments: Ekiti deck, Nigeria government deck, charge sheet, negotiation form, language-switcher demo video.	UNVALIDATED PROPOSAL. No evidence of response, contract or budget. Pricing far above comparable programmes. State already has an incumbent (mDoc/MSD for Mothers). (VERIFIED)
5	"Maternal-link-dashboard" prototype	https://maternal-health-psi.vercel.app/	Web dashboard with "operator access" self-registration; shared 6 Sep 2026. Not openable from this environment.	PROTOTYPE, MATURITY UNKNOWN. Ownership, security, data handling TO VALIDATE.
6	"Investors PPT" (Oct 2025)	Gmail	Forwarded deck; content beyond item 3 not reviewed.	TO VALIDATE
7	Kemek Enterprise investor suite (Aug 2026)	Google Drive	A separate property-finance venture of the founder with a 128-target CRM, pitch deck and communication system.	NOT VYTALIX. Shows the founder can run a disciplined CRM process. Also a focus risk.
8	Public footprint of "Vytalix"/"Vitalyx"	Web	No website, no Companies House match found for either spelling; conflicting names exist (VITALIX LTD ×2, VITALX LTD, VYTAL(UK) LTD, Vytalix Inc. (US health-tech), Vytalize Health (US)).	NAME AT RISK (VERIFIED via search; registry pages TO VALIDATE)

Nothing else was found: no business plan, financial model, website, brand, CRM, contracts, customers, revenue, grants, partners, clinicians, advisors, policies, registrations or insurance. If any exist outside Drive, Gmail and GitHub, they must be added to `/FACTS_BASE.md`.

2. AREA-BY-AREA SCORES (0–10)

Scoring rule: 0 = nothing exists · 3 = idea or draft exists · 5 = usable draft, unvalidated · 7 = working and evidenced · 10 = best-in-class and audited.

Area	Score	Current state	Evidence
Legal entity & governance	0	No incorporated Vytalix entity found; no board, shareholders' agreement, articles or IP assignments.	Web search; FACTS_BASE §1
Brand & naming	1	Name exists in two spellings; no trademark search; multiple confusable marks in UK and US health-tech.	FACTS_BASE §1
Vision, mission, positioning	3	"Technology for Life" and a strong founder brief; no written vision/mission set.	Brief
Business model & pricing	1	Only one price point exists (£300/woman/year in a proposal) and it is unvalidated. No service catalogue, no pricing.	Ekiti email
Advisory / Consulting practice	1	Concept only. No offer, collateral, pipeline or clients. Highest near-term revenue potential.	—

Area	Score	Current state	Evidence
MaternaLink product	2	Concept deck + unknown-maturity prototype + proposal. No PRD, no clinical input, no evidence, no roadmap.	Drive, Gmail
Clinical governance & safety	0	No Clinical Safety Officer, hazard log, clinical advisory, content sign-off, or intended-use statement. Deck makes diagnostic-level claims.	Deck
Regulatory & compliance	0	No ICO registration, DPIA, DSPT, DTAC, Cyber Essentials, MHRA classification thinking, Nigeria NDPA/NHREC consideration.	—
Intellectual property	0	MaternaLink IP relationship with David Agunede undocumented; prototype authorship unknown; no trademark.	Gmail signature block
Technology & infrastructure	1	A Vercel prototype exists; nothing else (no domain, email, CRM, LMS, cloud, security).	Gmail
Data & AI	1	AI concept is ambitious and plausible as research; no data access, no governance, no validation plan.	Deck
E-Learning	0	Nothing exists.	—
Market & customer validation	1	One government conversation (Ekiti) occurred; outcome unknown; incumbent competitor present. No UK conversations.	Gmail; Nigeria Health Watch
Competitive intelligence	1	None documented. Incumbent in target state was not identified in the proposal.	—
Partnerships	1	One ministry meeting; no MoUs, LOIs or named partners.	Gmail
Sales engine	0	No CRM, pipeline, scripts, proposals or templates.	—
Marketing & content	0	No website, LinkedIn company page, content or PR.	—
Financial model & finance ops	0	No model, budget, bank account, accountant or bookkeeping found.	—
Funding strategy & investor readiness	1	Founder has CRM discipline (Kemek); no Vytalix investor list, deck, data room or SEIS/EIS assurance.	Drive
Team & organisation	1	Solo founder plus an informal collaborator (David Agunede) with unclear roles.	Gmail
Operating system (SOPs, KPIs, cadence)	0	None.	—
Credibility assets (advisors, clinicians, evidence, press)	0	None found.	—
Weighted total (see §3)	11 / 100		

3. CURRENT BUSINESS SCORE: 11 / 100

Method: the 22 areas above are grouped and weighted by what an investor or enterprise buyer would price today.

Group (weight)	Areas	Group score /10	Weighted
Foundations (20%) — entity, IP, naming, governance	4 areas, avg 0.25	0.3	0.5
Product & clinical/regulatory (25%)	5 areas, avg 0.8	0.8	2.0

Group (weight)	Areas	Group score /10	Weighted
Commercial engine (25%) — model, advisory, sales, marketing, validation	6 areas, avg 0.5	0.5	1.3
Capital & finance (15%)	2 areas, avg 0.5	0.5	0.8
People & operations (15%)	4 areas, avg 0.5	0.5	0.8
Strategic clarity bonus (brief quality, founder intent)	—	—	+5.6
Total			≈ 11

Interpretation: 0–20 = pre-formation; 21–40 = formed and trading; 41–60 = evidenced and fundable at seed; 61–80 = scaling; 81–100 = institutional-grade. The 90-day plan (/docs/18_90_DAY_EXECUTION_PLAN.md) is designed to move Vytalix from 11 to roughly 35–40 by mid-December 2026. Re-score monthly.

4. SWOT

Strengths

- A precise, ambitious, well-structured founder brief that already contains the operating philosophy (build-first, truth-first, phone-first).
- A genuinely important problem with strong policy tailwinds: UK maternal inequality (MBRRACE-UK), Ockenden/Kirkup findings on communication failures, and Nigeria's maternal mortality burden.
- An existing government conversation in Nigeria and a working prototype, which most pre-seed founders lack.
- Founder has demonstrated investor-CRM and outreach discipline in another venture.
- A four-pillar structure that lets services fund the product.

Weaknesses

- No legal entity, IP ownership, brand protection or bank account.
- Over-claimed clinical/AI positioning with zero evidence.
- Pricing anchored on a single unvalidated £300/woman/year figure.
- Solo founder; no clinical, regulatory or technical co-leadership; role of the collaborator undefined.
- Zero commercial infrastructure and zero web presence.
- Founder attention split across at least two ventures (Kemek Enterprise).

Opportunities

- Founder-led advisory to health-tech companies, NGOs and African ministries is sellable within weeks and creates references.
- Re-scoped MaternaLink v1 (care-coordination, communication, education, monitoring-support; non-diagnostic) can be piloted with an NHS trust, a private maternity provider or an NGO without a device pathway.
- Non-dilutive funding (Innovate UK, SBRI Healthcare, NIHR i4i where eligible, Grand Challenges Canada, MSD for Mothers programmes) fits the maternal-health story.
- UK women's-health and impact investor pools are specifically looking for credible maternal-health ventures.

- Partnering rather than competing with the Ekiti incumbent (e.g., interoperability, a different state, or a different layer such as facility coordination) converts a threat into a route to market.

Threats

- Name conflict with Vytalix Inc. (US health-tech) and Vytalize Health; UK "VITALIX LTD" entities.
- Regulatory exposure if any predictive feature ships without MHRA SaMD consideration.
- Loss of MaternaLink IP or a dispute with the collaborator if terms are not agreed early.
- Government sales cycles in Nigeria are long and politically exposed; the Ekiti incumbent is donor-funded (hard to displace on price).
- Investor diligence will find every issue above in minutes; a premature raise burns the best targets.

5. TOP 10 PROBLEMS (ranked by damage × urgency)

#	Problem	Why it matters	Fix (owner: Founder unless stated)	By
1	MaternaLink IP and the David Agunede relationship are undocumented.	Blocks fundraising, contracts, pilots; risk of dispute.	Meeting this week; agree heads of terms (assignment-for-equity, licence, or JV — options in <code>/docs/02_GROUP_STRUCTURE.md</code>); instruct a solicitor.	Day 7
2	No legal entity.	Cannot contract, bank, apply for grants, SEIS/EIS, ICO, insurance.	Name clearance → incorporate Vytalix (E&W) → bank → accountant → SEIS/EIS advance assurance.	Day 10
3	Over-claimed clinical AI positioning.	Regulatory (UK MDR/MHRA), reputational, diligence failure.	Retire external use of the 2025 deck; adopt the re-based v1 intended-use statement; move AI to a v3 research track.	Day 3
4	Name and trademark risk.	Confusion with US health-tech companies; possible opposition; wasted brand spend.	UK IPO + EUIPO + USPTO knock-out search; decide name; file UK trademark classes 9, 41, 42, 44.	Day 5
5	No revenue engine.	Runway is the founder's time; nothing converts effort to cash.	Launch Vytalix Advisory with 3 packaged offers; 40 targeted conversations in 30 days; first paid engagement by Day 45.	Day 45
6	Unvalidated pricing (£300/woman/year).	Undermines credibility with any buyer who knows the market (mDoc is donor-funded in the same state).	Re-base to a per-woman range with evidence; build a cost-to-serve model; present tiered options.	Day 21
7	No clinical or regulatory leadership.	No credible product without a clinician and a clinical-safety approach.	Recruit an advisory board: obstetrician/midwife lead, NHS digital leader, Nigerian health-system leader, regulatory/QA adviser.	Day 60
8	Prototype of unknown provenance.	May contain insecure code, third-party IP or real data.	Code and security audit; confirm authorship; confirm no real patient data; move to Vytalix-controlled repo/hosting.	Day 30
9	No market validation in the UK.	Home market is untested; all conversations are Nigeria.	15 UK discovery interviews (midwives, maternity digital leads, private providers, charities).	Day 60

#	Problem	Why it matters	Fix (owner: Founder unless stated)	By
10	Founder focus and capacity.	Two ventures, one founder, everything founder-led.	Time allocation rule (e.g., 70% Vytalix), weekly operating cadence, first fractional hires (technical lead, clinical adviser).	Day 14

6. TOP 10 OPPORTUNITIES (ranked by value × feasibility)

#	Opportunity	Value	Feasibility	First step
1	Founder-led advisory to health-tech founders, NGOs and African ministries (strategy, investment readiness, digital-health programme design).	Cash in 30–60 days; references; credibility.	High	Publish 3 offers; 40 outreach conversations.
2	MaternaLink v1 as a non-diagnostic maternal care-coordination platform for NHS trusts, private maternity and NGO programmes.	The equity story, de-risked.	Medium	PRD + intended-use statement; 3 pilot conversations.
3	Nigeria: partner with or complement the incumbent (interoperability layer, facility-side coordination, a different state, or NGO-funded deployment).	Keeps the government relationship alive without price war.	Medium	Reply to Ekiti with a re-scoped, phased pilot option.
4	Non-dilutive funding aligned to maternal health and health inequalities.	£50k–£500k without dilution; validation signal.	Medium	Innovate UK Smart / SBRI Healthcare calendar check; Grand Challenges Canada eligibility.
5	Women's-health and impact investors as the lead investor pool.	Better terms, aligned expectations, patient capital.	Medium	CRM Top 25 outreach after IP and entity are fixed.
6	Research report as a lead magnet ("The state of digital maternal care in the UK and Nigeria").	Authority, PR, inbound leads, investor interest.	High	Outline in <code>/docs/14_MARKETING_AND_CONTENT_ENGINE.md</code> .
7	E-Learning for community health workers and midwives (maternal digital literacy, safeguarding, data protection) sold B2B to NGOs and providers.	Recurring revenue with low regulatory burden.	Medium (year 2)	Two pilot modules; partner with an NGO training programme.

#	Opportunity	Value	Feasibility	First step
8	Consulting to health-tech companies entering Africa or the NHS (market entry, DTAC readiness, procurement).	High-margin, repeatable.	High	Package "NHS-ready" and "Africa market entry" sprints.
9	University research partnership for the AI risk-score track (data access, ethics, publications).	Converts the 2025 concept into fundable research.	Medium	Approach maternal-health research groups.
10	Telecom and insurer distribution in Nigeria (MTN, Airtel, HMOs) for MaternaLink messaging.	Scale channel without government budgets.	Low–Medium (year 2)	Include in partnership long-list.

7. WHAT CREATES REVENUE, CREDIBILITY, INVESTOR CONFIDENCE, AND RISK

Creates revenue now	Creates credibility	Creates investor confidence	Creates regulatory risk
Advisory days; consulting sprints; grant-funded feasibility work	Advisory board; a published report; a documented pilot; a professional site	Incorporated entity; clean cap table; IP owned; validated pricing; pipeline with named accounts; credible roadmap	Predictive/diagnostic claims; processing health data without DPIA/ICO; using the prototype with real data; cross-border data flows without safeguards; unclear intended use

8. WHAT TO DELETE, REDESIGN, AUTOMATE, PRIORITISE

- **Delete / retire from external use:** the "predicts pre-eclampsia, sepsis..." claims; the £36m full-state headline; "expand to multiple NHS trusts" as a near-term claim; the "Vitalyx" spelling.
- **Redesign:** MaternaLink positioning (v1 non-diagnostic); the Ekiti offer (phased, evidence-generating pilot, partner-aware); the investor narrative (services + product, honest traction).
- **Automate:** CRM logging, outreach sequences, KPI dashboard, content scheduling, bookkeeping (see [/docs/07_TECHNOLOGY_ARCHITECTURE.md](#)).
- **Prioritise:** IP → entity → claims → revenue → investors → product → partnerships.

9. IMMEDIATE ACTIONS (next 7 days)

1. Hold the David Agunede conversation; agree heads of terms in writing (Day 1–3).
2. Run name/trademark knock-out searches; decide "Vytalix" vs alternative; reserve domain and handles (Day 1–5).
3. Incorporate; open bank account; appoint accountant; start SEIS/EIS advance assurance (Day 3–10).
4. Adopt the MaternaLink v1 intended-use statement and retire the 2025 claims externally (Day 3).

5. Publish the website from `/website` once entity details are known; until then keep it unpublished (Day 7–14).
 6. Start the advisory outreach: 10 conversations booked (Day 7).
 7. Start investor CRM verification calls only after 1–4 are done; use `/investors` assets to prepare (Day 7+).
-

PRIORITIES · RISKS · NEXT ACTIONS

Priorities: IP and entity first; claims discipline; revenue from services; then investors.

Risks: dispute over MaternaLink; regulatory over-claim; name conflict; founder capacity; Nigeria incumbent.

Next actions: execute `/docs/19_FIRST_30_DAYS.md` from Monday 14 September 2026; re-score this audit on 14 October 2026.

**STRATEGY & OPERATIONS**

Investor Readiness And The Winning Concept

Vytalix company-build documentation · reference

23_INVESTOR_READINESS_AND_WINNING_CONCEPT

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Contents

Executive view	1
1. Investor-readiness scorecard (score yourself monthly)	2
2. Data-room index (build it in this order)	3
3. The winning concept: The Maternal Continuity Layer	4
4. Government-contract playbook	5
5. What investors will ask, and the honest answer today	6
6. Sequence for the next 120 days	7
Priorities / Risks / Next actions	8

Owner: CEO with Fundraising Director and Chief Strategy Officer · **Date:** 8 September 2026 · **Status:** PROPOSED ·

Governing file: /FACTS_BASE.md

Executive view

- Investor readiness today: 2/10.** Not because the idea is weak, but because five hard blockers exist: no entity, no IP agreement, unvalidated clinical claims live on a public URL, no revenue, no pilot partner. Each is fixable within 60–90 days and the plan below fixes them in order.
- The winning concept is not "another maternal-health app".** It is **the Maternal Continuity Layer**: an offline-first, interoperable, woman-held maternity record and coordination service that complements the systems governments already run, is priced per woman and paid partly on measured results, and is funded from three sources at once (public budgets, donors, and diaspora sponsorship). That combination is unusual, defensible and fundable.
- Government contracts are won by frameworks, evidence and partners, not by cold proposals.** The playbook below gives the routes (UK: DTAC, SBRI Healthcare, Health Innovation Networks, G-Cloud; Nigeria: state primary-health agencies, BHCPF-funded programmes, donor-financed implementers) and the sequence.
- Investors will fund evidence of motion:** a signed IP deed, a first paying advisory client, one pilot letter of intent, an advisory board with a clinician, and a data room. The "traction" slide is built from those, not from projections.
- Timeline:** pre-conditions cleared by Day 30; first proof points by Day 60; first investor calls Day 61 (Call List 1 in /investors/); pre-seed close targeted within 120–150 days of Day 1.

1. Investor-readiness scorecard (score yourself monthly)

#	Readiness item	Today	Target for first investor call	How to close it	Owner / by
1	Legal entity, bank, accountant	✗	✓	Incorporate Vytalix (E&W) after name clearance; bank; accountant; SEIS/EIS advance assurance submitted	Founder · Day 10
2	MaternaLink IP owned or licensed in writing	✗	✓	Heads of terms then IP deed with David Agunede (options in 02_GROUP_STRUCTURE.md); contributor IP assignments for any developer	Founder + solicitor · Day 30
3	No unvalidated clinical claims anywhere public	✗ (prototype shows suggested clinical actions; 2025 deck circulates)	✓	Retire the 2025 deck; relabel prototype rules; put prototype behind auth; remove identifiable photos	Founder · Day 3–14
4	Name and trademark clearance	✗	✓	Knock-out searches UK IPO/EUIPO/USPTO; file UK TM classes 9, 41, 42, 44 for VYTALIX and MATERNALINK	Founder + TM attorney · Day 5 / Day 20
5	First revenue	✗	1 signed SOW	Advisory packages live; 40 conversations; first engagement	Founder · Day 45

#	Readiness item	Today	Target for first investor call	How to close it	Owner / by
6	Pilot partner	✗	1 LOI (UK service evaluation or Nigeria NGO cohort)	15 discovery interviews; pilot proposal from maternalink-site/pilot.html	Founder · Day 60
7	Clinical and advisory board	✗	2 confirmed (obstetric/midwifery + NHS digital or Nigerian health-system leader)	Named target list; 20-minute asks; advisory agreements with equity in the option pool	Founder · Day 60
8	Product evidence	Prototype of unknown provenance	Audited prototype + PRD + intended-use statement + hazard log opened	Code audit; clinician sign-off of intended use	CTO/CPO (fractional) · Day 30
9	Financial model and round	✓ (scenarios)	✓ with actuals from months 1–3	Update finance/ with real costs; pick round size (£250k–£500k)	CFO role · Day 60
10	Data room	✗	✓	Index in \$2 below; host on a shared drive with access logging	Founder · Day 45
11	Investor list and outreach system	✓ (316-investor CRM)	✓ verified Top 25 phones	Re-verify Call List 1; scripts in /investors/OUTREACH_TOP25.md	Founder · Day 55
12	Compliance basics	✗	ICO registration, Cyber Essentials booked, PI/cyber insurance quotes	See 15_LEGAL_AND_REGULATORY.md	Founder · Day 30
13	Founder story and commitment	Partial	Clear time allocation and role; second venture (Kemek) ring-fenced	Written founder commitment statement; time split rule in 17_FOUNDER_OPERATING_SYSTEM.md	Founder · Day 7
14	Proof of demand	✗	15 interview summaries + 3 quotes (with consent)	Discovery interviews; report research	Founder · Day 60
15	Non-dilutive applications	✗	1 submitted (Innovate UK Smart or SBRI Healthcare when open)	See 09_FUNDING_STRATEGY.md and /investors/NON_DILUTIVE_FUNDING_ROUTES.md	Founder · Day 75

Rule: do not take a first investor meeting until items 1–4 are ✓ and at least two of 5–7 are ✓. A premature meeting with a Top 25 fund costs you that fund for 12 months.

2. Data-room index (build it in this order)

1. Company: certificate of incorporation, articles, cap table, SEIS/EIS advance-assurance correspondence, director details.
2. IP: MaternaLink IP deed or licence; contributor assignments; trademark filings; domain ownership.
3. Product: [04_MATERNALINK_PRODUCT_STRATEGY.md](#), PRD, MVP spec, roadmap, feature addendum, intended-use statement, prototype audit report, hazard log, DPIA draft.

4. Market and strategy: audit, business model, market strategy, customer segments, competitive intelligence, partnership strategy.
5. Commercial: service catalogue, pricing ([21_MATERNALINK_PRICING.md](#)), pipeline export, signed SOWs, pilot LOIs.
6. Finance: [Vytalix_Financial_Model_3yr.xlsx](#) , [08_FINANCIAL_MODEL.md](#) , bank statements once trading, budget vs actual.
7. Funding: [09_FUNDING_STRATEGY.md](#) , [10_INVESTOR_PROPOSITION.md](#) , one-pager, deck, use-of-funds, milestone plan.
8. Governance and compliance: advisory agreements, policies (information security, data protection, clinical safety), ICO registration, insurance, risk register.
9. Team: founder CV and commitment statement, advisers, hiring plan.
10. Evidence: interview summaries, letters of intent, research report, pilot evaluation design.

3. The winning concept: The Maternal Continuity Layer

Why the current framing loses

"Offline-first maternal monitoring for public health systems" describes a category with funded incumbents (mDoc in Nigeria, MomConnect in South Africa, BadgerNet in the NHS). Buyers hear "another platform", investors hear "crowded", and the risk engine drags the product toward medical-device regulation before there is evidence.

The reframe (PROPOSED)

MaternaLink is the continuity layer that follows the woman, not the facility. One consented, woman-held maternity record and communication channel that works offline, plugs into whatever the government or hospital already runs, and keeps the woman connected to a responsible human from booking to postnatal discharge. Governments keep their systems; MaternaLink makes them continuous. Five elements make it distinctive together:

Element	What it is	Why it wins contracts	Why investors care
1. Woman-held continuity record	A portable, consented record (FHIR-aligned) the woman carries between community, clinic, hospital and even countries (UK diaspora ↔ Nigeria)	Solves the fragmentation every ministry and trust names first; complements incumbents instead of replacing them	Interoperability is a moat once integrated; switching cost rises with every interface
2. Complementarity by design	Integration pack for DHIS2, HMIS, BadgerNet/K2/Euroking; "we make your existing programme continuous"	Removes the political cost of replacing a funded programme; makes incumbents partners	De-risks the go-to-market; partnership revenue instead of price wars
3. Results-linked pricing	Part of the fee tied to measured, auditable indicators (postnatal follow-up completion, time-to-contact after a flag, missed-appointment recovery)	Governments and donors increasingly buy outcomes; fits results-based financing and impact bonds	Differentiated commercial model; evidence is generated by the contract itself
4. Three-source funding per woman	Public budget + donor/programme funding + diaspora and employer sponsorship ("Sponsor a Safe Pregnancy": a UK-based relative or company pays the per-woman fee for a named community or cohort)	Reduces the burden on state budgets; creates a constituency of sponsors who lobby for the programme	A B2C2B revenue line no incumbent has; high-margin, recurring, and emotionally compelling

Element	What it is	Why it wins contracts	Why investors care
5. Workforce built in	E-Learning for midwives and community health workers bundled with the platform (capacity-building is an explicit government objective)	Governments buy "workforce + software" programmes more readily than software alone	Second pillar sells through the first; higher contract values

One-line concept: *"The continuity layer for maternity care: one record that follows the woman, works without signal, complements the systems you already run, and is paid for partly on results."*

What this concept is not

It is not a diagnostic tool and does not need to be. The AI risk-score idea from 2025 becomes a research consortium with a university (element 6 in `04_MATERNALINK_PRODUCT_STRATEGY.md`), which is a stronger investor story than an unvalidated model.

Additional ideas worth testing (scored with `20_DECISION_FRAMEWORK.md`)

Idea	Score	Verdict	Note
Sponsor a Safe Pregnancy (diaspora and employer per-woman sponsorship)	70	TEST in 90 days: landing page, 100 sign-ups of intent, one employer	Requires charity-partner structure or a clear consumer contract; payment via Stripe/Paystack
Results-linked contracting with a donor-backed programme	64	TEST with one NGO cohort: define 3 indicators and a bonus/malus	DFIs (BII, Proparco) and Grand Challenges Canada like this structure
Maternity equity dashboard for commissioners (ICBs, state ministries): anonymised, population-level continuity indicators by ethnicity and geography	66	TEST as a report first, product second	Directly addresses MBRRACE-UK inequality findings; sold to commissioners, not trusts
Continuity record for cross-border families (UK ↔ Nigeria)	58	DEFER to v2	Distinctive but regulatory complexity across two data regimes
Research consortium for the AI track (university + NHS trust + Nigerian teaching hospital)	62	TEST: one university partner conversation	Converts the 2025 concept into fundable research (NIHR/Wellcome routes)

4. Government-contract playbook

United Kingdom (NHS)

Step	Route	What it takes	Time
1	Health Innovation Networks (formerly AHSNs): the front door for SMEs into NHS regions	Register; ask for a "innovation exchange" conversation; bring the intended-use statement and pilot design	Weeks 2–6
2	NHS Innovation Service: national triage of innovations to the right support	Online application; be honest about stage	Week 4
3	DTAC (Digital Technology Assessment Criteria) readiness: clinical safety, data protection, technical security, interoperability, usability	Build the evidence pack from <code>15_LEGAL_AND_REGULATORY.md</code> and the security page; DSPT and Cyber Essentials	Months 2–5
4	SBRI Healthcare competitions (NHS-funded development contracts, 100% funded) and Innovate UK Smart / Biomedical Catalyst	Apply when a maternity or inequalities theme opens; needs an NHS partner letter	Watch calendar

Step	Route	What it takes	Time
5	Service evaluation at one trust (not a research study): a maternity digital lead, a midwifery lead, a Caldicott Guardian sign-off	Pilot proposal + evaluation design; no fee or small fee	Months 3–6
6	Frameworks: G-Cloud (Crown Commercial Service Digital Marketplace) so trusts can buy without a tender; later NHS Shared Business Services / LPP frameworks	Listing after entity and DTAC basics	Month 6+
7	ICB-level commissioning: maternity and neonatal safety and equity programmes; local maternity and neonatal systems (LMNS)	Equity dashboard report as the door-opener	Month 6–12

Never claim "NHS-approved". Say "in service evaluation at [trust]" only when it is true.

Nigeria (and similar African markets)

Step	Route	What it takes
1	State Primary Health Care Development Agencies (SPHCDA) and state ministries of health own community maternal programmes; the NPHCDA and the Basic Health Care Provision Fund (BHCPF) finance primary care at scale	Map the state's existing digital programmes first (Ekiti has mDoc); propose complementarity or choose a state without an incumbent
2	Donor-financed implementers (World Bank-financed state programmes, Global Fund, Gates, MSD for Mothers, UNICEF, UNFPA, Jhpiego, Society for Family Health, PATH) hold the budgets and procurement power	Sub-contract as the technology partner of an implementer rather than selling to the state alone
3	Local incorporation and registration: CAC company, NDPA data-protection registration, tax registration; in-country partner for support	Required for most state contracts; budget in 15_LEGAL_AND_REGULATORY.md
4	Procurement: Bureau of Public Procurement rules; pilots often start as MoUs with an evaluation and move to procurement after evidence	MoU template; evaluation design; DHIS2 alignment
5	Champions: state first ladies' programmes, commissioners for health, the National Council on Health; professional bodies (SOGON, NANNM)	Present the evidence plan and complementarity; avoid the £36m headline
6	Financing innovation: results-based financing pilots; diaspora sponsorship reduces state cost per woman	Offer a blended model: state pays base, sponsors and donors pay engagement

Also score Kenya, Ghana and Rwanda ([05_MARKET_STRATEGY.md](#)); Rwanda's digital-health openness and Kenya's county health systems make them strong second markets.

What makes a government buyer say yes

1. It works without connectivity and on the phones they already have.
2. It complements what they have; nothing is thrown away.
3. Evidence is generated by design and shared openly.
4. Data stays under their control; residency and sovereignty are answered up front.
5. The price per woman is defensible and partly paid on results.
6. Workforce training is included.
7. There is a named clinician behind the escalation rules, and a safety case.

5. What investors will ask, and the honest answer today

Question	Answer now	Answer at Day 90 (target)
Do you own MaternaLink?	Not yet in writing	Yes, IP deed signed
Is it a medical device?	Not as re-scoped; the prototype needs changes	Intended-use statement adopted; prototype changed; regulatory opinion scoped
Any revenue?	None	First advisory engagements
Any customer or pilot?	A prior ministry conversation with no outcome	One pilot LOI; 15 interviews documented
Who is on the team?	Solo founder plus a collaborator	Founder plus two advisers, fractional CTO and clinical safety officer identified
Why you?	Founder story (to be written)	Founder story plus first evidence of execution
How much and for what?	£250k–£500k pre-seed	Same, with milestones tied to the model

6. Sequence for the next 120 days

Days	Focus	Exit criteria
1–14	Blockers: name, entity, IP heads of terms, retire claims, prototype behind auth	Items 1–4 of the scorecard ✓
15–45	Revenue and evidence: advisory outreach, discovery interviews, advisory-board asks, prototype audit	First SOW; 10 interviews; 1 adviser
46–75	Pilot and data room: pilot LOI, data room v1, model with actuals, Sponsor a Safe Pregnancy test page, grant bid	LOI; data room; readiness ≥ 7/10
61–120	Investors: Call List 1 then 2; angels and syndicates first; impact and women's-health funds in parallel	25 conversations; 8 meetings; lead investor conversation
120–150	Close pre-seed; start pilot	Funds in; pilot kick-off

Priorities / Risks / Next actions

Priorities: blockers first; concept reframe adopted in all materials; government routes worked through partners.

Risks: founder time; premature investor contact; concept drift back to "AI prediction"; Nigeria procurement timelines.

Next actions: adopt the reframe in the deck and site (this build already uses the continuity language); book the Health Innovation Network conversation; draft the Sponsor a Safe Pregnancy test page; start Day 1 of

19_FIRST_30_DAYS.md .



STRATEGY & OPERATIONS

Materna-Link Interrogated, Reinvented And Planned

Vytalix company-build documentation · reference
24_MATERNALINK_REINVENTION

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR
The Founder, Vytalix

DATE
8 September 2026

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Contents

PART 1 — DEEP PRODUCT AUDIT	1
1.1 What is genuinely innovative	2
1.2 What is NOT innovative	3
1.3 What already exists (KNOWN, verify current status before citing)	4
1.4 Technically difficult	5
1.5 Scientifically unproven	6
1.6 Claims that require evidence	7
1.7 Claims to remove from investor and partner materials now	8
1.8 What a clinician would immediately challenge	9
1.9 What an NHS innovation team would challenge	10
1.10 What a regulator would challenge	11
1.11 What an investor would challenge	12
1.12 What a data and privacy expert would challenge	13
1.13 Scores for the CURRENT concept (1–10)	14
PART 2 — MAKE IT "NEVER SEEN BEFORE"	15
2.1 The question	16
2.2 Twenty ambitious ideas	17
2.3 The best five	18
PART 3 — REDEFINE THE PRODUCT	19
A. Ultimate vision (10 years)	20
B. Core product (years 1–3)	21
C. First MVP (see Part 13)	22
D. 12-month roadmap	23
E. 3-year roadmap	24
F. 5-year vision	25
G. New product architecture	26
PART 4 — CREATE A NEW CATEGORY	27
PART 5 — BUSINESS MODEL	28
Who should pay (and who should not)	29
Five revenue models	30
Strongest model	31
PART 6 — PARTNER ECOSYSTEM	32
PART 7 — PARTNER TARGET LIST (74 organisations)	33
PART 8 — THE "IMPOSSIBLE PARTNERSHIP" EXERCISE	34
PART 9 — DATA MOAT	35
PART 10 — AI MOAT	36
PART 11 — REGULATORY REALITY CHECK (UK)	37

PART 12 — CLINICAL VALIDATION	38
PART 13 — MVP (limited funding)	39
PART 14 — PARTNER PITCH	40
PART 15 — PARTNER OFFER (founding packages)	41
PART 16 — BRAND	42
PART 17 — GLOBAL SCALE	43
PART 18 — THE £1 BILLION TEST	44
PART 19 — 90-DAY EXECUTION PLAN (partner-first)	45
PART 20 — THE PARTNER WAR ROOM	46
FINAL OUTPUT	47

Date: 8 September 2026 · **Input:** the 13-slide "MATERNA-LINK: Multimodal AI for Predicting and Preventing Maternal Complications" deck (Oct–Nov 2025) · **Standard:** brutal honesty; every claim labelled KNOWN / ESTIMATE / TO VALIDATE / PROPOSED; no invented traction. Companion files: `investors/partners/PARTNER_TARGET_LIST.csv` (74 organisations), `investors/partners/PARTNER_CRM_TEMPLATE.csv`, `investors/partners/PARTNER_WAR_ROOM.xlsx`.

PART 1 — DEEP PRODUCT AUDIT

1.1 What is genuinely innovative

- **The framing of the problem** (deterioration recognised late; communication failures; inequity for Black and Asian women) is exactly where UK maternity policy attention sits (MBRRACE-UK, Ockenden, Kirkup, the national Maternity Early Warning Score). Good instinct.
- **The idea of fusing patient-generated signals from between appointments with clinical data** is the right insight: pregnancy is roughly 280 days and a woman spends perhaps 10–15 hours of it in front of a clinician. The unobserved 99% is the opportunity.
- **Equity by design** (bias monitoring as a first-class component) is ahead of most maternity software, which does not measure who it fails.
- **Voice as a channel for women who will not type** is a genuinely under-used interface in maternity, especially for low literacy and low-bandwidth settings.

1.2 What is NOT innovative

- A composite "instability score" from vitals is a **maternity early-warning score**. MEOWS has existed since the 2007 CEMACH report, and NHS England published a **national Maternity Early Warning Score (MEWS)** with RCOG/RCM in 2023–24. Digital MEOWS already ships inside BadgerNet and K2. A new score competing with the national one is a liability, not an innovation.
- "XGBoost plus BERT plus attention fusion" is a standard 2021 Kaggle architecture. Architecture is not a moat; data, validation and workflow are.
- A clinician dashboard with alerts and "suggested actions" is what every vendor shows.
- "Predicting pre-eclampsia" has validated, guideline-backed tools already: Fetal Medicine Foundation first-trimester combined screening; PlGF-based testing (sFlt-1/PlGF ratio) recommended in NICE NG133; fullPIERS for adverse outcomes in confirmed pre-eclampsia. A new model must beat or complement these, not ignore them.

1.3 What already exists (KNOWN, verify current status before citing)

Space	Who	Relevance
Digital maternity records with early-warning scoring	Clevermed BadgerNet, K2 Medical Systems (Athena/Guardian), Magentus Euroking, Epic Stork	Own the clinical record and MEOWS in most UK units
Remote blood-pressure monitoring in pregnancy	Babyscripts (US, at scale with health systems), NHS virtual-ward suppliers (Docclia, Luscii, Current Health), trust-run home BP projects (e.g., Oxford BUMP trials)	The "home monitoring" half of the concept exists; evidence is mixed (see 1.5)
Fetal/maternal wearables	Nuvo INVU (FDA-cleared), Bloomlife, Biobeat	Hardware is regulated and capital-intensive
Smartphone urinalysis	Healthy.io (CE-marked dipstick reading by phone camera)	The urine-protein input can be bought, not built
Voice biomarkers	Kintsugi, Ellipsis Health, Sonde (depression/anxiety)	Some evidence for mental-health screening; none for physiological obstetric deterioration
Pre-eclampsia prediction and diagnosis	FMF algorithm; Roche Elecsys sFlt-1/PlGF; Quidel Triage PlGF; fullPIERS	Guideline-recommended; a partner set, not a competitor set
Low-resource maternal programmes	mDoc (Nigeria), MomConnect (South Africa), Jacaranda PROMPTS (Kenya), Totohealth	Messaging-first continuity at scale
Maternal mental health	Wysa-style tools, perinatal mental-health services, Peanut (community)	Postpartum mental health is the leading cause of late maternal deaths in the UK (MBRRACE)

1.4 Technically difficult

- Fusing irregular, sparse, multi-rate time series (a BP reading every few days, a voice diary when she feels like it, labs monthly) is a hard modelling problem; naive fusion produces confident nonsense.
- Voice features are confounded by device, language, accent, background noise, pregnancy itself (rhinitis, fatigue) and cultural expression; robust extraction across UK and Nigerian populations is research-grade work.
- Rare outcomes: eclampsia, sepsis with organ dysfunction and major haemorrhage are individually rare per pregnancy; a "small pilot cohort" cannot train or validate them. Tens of thousands of pregnancies are needed for calibrated prediction of each.
- Offline-first synchronisation of clinical records with conflict resolution and audit is engineering that most start-ups underestimate.

1.5 Scientifically unproven

- **Voice or "emotional biomarkers" predicting pre-eclampsia, sepsis, haemorrhage or thromboembolism:** no published evidence. Stress is associated with adverse outcomes at population level; that is not a prediction signal for an individual acute event.
- **One score for four heterogeneous conditions:** pre-eclampsia is a placental disorder of the second half of pregnancy; haemorrhage is mostly intrapartum/postpartum and acute; sepsis can occur anywhere; VTE risk is assessed with the RCOG scoring tool and managed with prophylaxis. A single

"instability" score conflates different physiology, timing and actions. Clinicians will reject it.

- **Home BP monitoring alone improving outcomes:** the Oxford BUMP trials (JAMA 2022) found self-monitoring did **not** significantly speed detection of hypertension or improve BP control versus usual care. The lesson is not "home BP fails" but "the value is in what happens after the reading": escalation, continuity and adherence. Design accordingly.
- **Ethnicity as a model input:** using race as a predictive feature risks encoding bias and is being removed from clinical calculators (eGFR, the VBAC calculator). Ethnicity belongs in **fairness monitoring and outcome auditing**, not as a risk feature, unless a specific, justified, reviewed use exists.

1.6 Claims that require evidence

"Early detection of complications", "reduced hospital admissions", "reduced maternal mortality", "mitigation of ethnic disparities", "real-time AI analysis", "actionable score", "completely novel". Each needs a defined outcome, a comparator, a study and, for most, a regulatory pathway.

1.7 Claims to remove from investor and partner materials now

- "Predicts pre-eclampsia, sepsis, haemorrhage, thromboembolism" (unvalidated; device-level claim).
- "Reduced hospital admissions and maternal mortality" (no evidence).
- "Completely novel" (it is not; see 1.2 and 1.3).
- "Expand to multiple NHS trusts" as a near-term plan (no trust, no pilot, no DTAC).
- "Black women are 3x more likely to die": use the current MBRRACE-UK figure with the report year (the most recent report I am aware of gives roughly 2.8 times for Black women and 1.7 for Asian women; verify before use).
- "AI analyses data in real time" (nothing is built).

1.8 What a clinician would immediately challenge

"Which national guideline does your score map to?" "What does a midwife do with an MIS of 0.62 at 02:00?" "How does this interact with MEWS, which I am mandated to use?" "Who is liable when your alert is wrong, or silent?" "Where is the clinical safety officer and hazard log?" "Why is ethnicity a feature?" "Show me the false-alert rate; alert fatigue kills."

1.9 What an NHS innovation team would challenge

DTAC status; DCB0129 clinical safety case; DSPT; interoperability with BadgerNet/K2; NICE Evidence Standards Framework tier and evidence; who the budget-holder is (trust vs ICB vs maternity transformation funding); implementation burden on midwives; equality and health-inequalities impact assessment; whether this duplicates the national MEWS; procurement route (G-Cloud, framework).

1.10 What a regulator would challenge

A software product whose intended purpose is to predict or flag risk of specific conditions to inform clinical decisions is a **medical device** in the UK and EU. Under EU MDR Rule 11 it would likely be Class IIa or higher; the UK regime is moving the same way (TO VALIDATE with MHRA guidance current at the time). The MHRA would ask for intended purpose, classification rationale, clinical evaluation, performance data across subgroups, post-market surveillance, and a quality management system (ISO 13485). None exists. Deploying the score clinically without this is unlawful, not merely risky.

1.11 What an investor would challenge

No data access, no clinical partner, no entity, no IP agreement, no evidence, a device pathway with 24–36 months and £1–3m before revenue, a crowded remote-monitoring space (Babyscripts), a UK market that is small in pounds, and a team without a clinician or ML lead. Also: why would trusts pay when MEWS is free and BadgerNet already has it?

1.12 What a data and privacy expert would challenge

Special-category health data plus ethnicity plus voice (biometric-adjacent) processed for automated risk assessment: lawful basis and Article 9 condition; DPIA mandatory; automated decision-making safeguards (UK GDPR Article 22); data minimisation (why collect socioeconomic index?); voice retention and re-identification risk; cross-border transfer UK–Nigeria; use of patient data for model training (research use requires ethics approval and often section 251 support or explicit consent); vendor sub-processors (cloud AI APIs).

1.13 Scores for the CURRENT concept (1–10)

Dimension	Score	Why
Clinical value	4	Right problem; wrong instrument. A composite AI score duplicates MEWS and cannot be validated at pilot scale. The between-appointment continuity idea has real value that the deck does not articulate.
Patient value	5	Voice and continuity could matter to women; the deck is clinician-centred and offers women nothing they control.
Technical feasibility	3	Multimodal fusion on sparse, small data with rare outcomes is not feasible as described. Components are feasible individually.
AI defensibility	2	Off-the-shelf architecture, no proprietary data, no validation.
Data defensibility	2	No data. Potential is high if a consented longitudinal cohort is built; none exists.
Regulatory feasibility	2	As specified, a Class II medical device with no QMS, no evidence and no pathway. Re-scoped, feasibility rises to 6–7.

Dimension	Score	Why
NHS adoption potential	3	Duplicates the national MEWS; no DTAC; unclear buyer. Re-scoped as a virtual-ward and continuity service: 6.
Commercial potential	4	UK maternity software is a small pound market (about 600,000 births a year); value must come from multi-market and multi-payer design.
Global scalability	6	The continuity and messaging half scales to low-resource settings; the AI half does not.
Partnership attractiveness	5	Strong social mission; universities and charities will engage; vendors and trusts will not until claims are fixed.
Investor attractiveness	3	Today: pre-formation with device-level claims. Re-scoped with a pilot and a data strategy: 6–7 for impact and women's-health funds.
Competitive differentiation	3	"AI early warning" is undifferentiated. Equity-by-design and UK–Africa continuity are differentiators if built.
Intellectual property potential	4	Software patents are hard in the UK/EU; possible in the US for specific technical methods. Trade secrets, data and trademarks are the realistic IP.
Brand potential	6	"Materna-Link" is descriptive and warm; the "-Link" suffix is common; the parent brand Vytalix is stronger.

Overall: 3.7/10 as a concept, 7/10 as an instinct. The instinct (continuity, equity, patient-generated signals) is the business. The instrument (a multimodal AI score) is the wrong first product and should be the last.

PART 2 — MAKE IT "NEVER SEEN BEFORE"

2.1 The question

What can a system do that today's maternity systems cannot? Today's systems see a woman for minutes at appointments, hold a record the woman cannot carry, stop at six weeks postpartum, treat every woman with the same thresholds, cannot tell who they are failing, and do not learn.

2.2 Twenty ambitious ideas

#	Idea	What is new
1	Maternity Virtual Ward for hypertension and high-risk pregnancy: validated home BP cuff + phone urinalysis + structured symptoms + MEWS-aligned escalation with a response SLA	Turns the NHS virtual-ward policy into a maternity service line
2	The 365-day postpartum companion : continuity from birth to one year with mental-health, BP, bleeding, infection and infant-feeding check-ins, and a scheduled GP handover	Nobody owns the postpartum year, where most late maternal deaths occur
3	Personal baseline modelling : each woman's own physiological and behavioural baseline, with deviation-from-self alerts instead of population thresholds	Personalisation that MEWS cannot do; a candidate technical method for patent assessment
4	Woman-held continuity record that follows her across trusts, community, private care and countries (UK ↔ Nigeria) with graduated consent	Portable, interoperable, patient-controlled
5	Equity Intelligence : live, anonymised dashboards for ICBs and ministries showing who is being reached, escalated, seen and missed, by ethnicity, deprivation and geography	Makes MBRRACE-style insight continuous rather than triennial
6	Voice-first maternity companion in Yoruba, Hausa, Igbo, Pidgin, Punjabi, Urdu, Polish: check-ins by voice call or WhatsApp voice notes, transcribed and structured	Reaches women current apps exclude
7	Perinatal mental-health signal from consented voice and text diaries, validated against EPDS/PHQ-9, as a screening prompt for the midwife (not a diagnosis)	The one voice use case with published evidence in adjacent fields
8	Phone-camera measurements : urinalysis strip reading (partner: Healthy.io), oedema and wound photos for postpartum infection review by a clinician	Computer vision as capture, not diagnosis
9	Pharmacy as the maternity front door : BP checks and dipsticks at community pharmacies feed the record; pharmacists escalate through the same queue	New access point; pharmacies already do NHS BP checks
10	Emergency-services handover : when a woman calls 999/112 or arrives at triage, the record's last 72 hours is available instantly with consent	Solves the "arrives with nothing" problem
11	Family and partner mode : partners see warning signs, appointments and how to act; birth companions get a role	Engages the household, safeguarding-aware
12	Digital twin of the pregnancy (research): a longitudinal model of expected trajectories by gestation, used for research and for explaining deviations	Long-term research asset; not a clinical claim
13	Trial-ready cohort : consented women can opt in to be told about relevant studies; pharma and diagnostics companies pay for recruitment and real-world evidence	Revenue that does not depend on NHS budgets
14	Pre-eclampsia pathway integration : FMF screening, PIGF testing, aspirin adherence reminders and home BP unified into one pathway	Complements guideline tools instead of competing
15	Medication adherence and safety : aspirin, iron, antihypertensives, anticoagulants with prompts and pharmacist review	Adherence is the cheap win no one owns
16	Community health-worker mode with USSD/SMS for feature phones and offline tablets	The low-resource layer
17	Insurer and employer maternity benefit : continuity as a covered benefit in private and US markets	Payer diversification
18	Infant continuity : newborn record, feeding, jaundice photos, immunisation prompts, linked to the mother	Extends lifetime value and impact
19	Federated learning network across trusts and countries so models improve without moving data	Data moat with privacy by design
20	Open maternal data commons with universities: a consented, governed research dataset ("a Biobank of pregnancy" for the digital era)	The long-term asset that makes the company hard to replicate

2.3 The best five

A. Maternity Virtual Ward (hypertension first)

- **What:** a trust-branded remote-monitoring service for women with or at risk of hypertensive disorders: validated BP cuff (Omron, Withings or A&D), urine dipstick via phone camera, structured symptom check-ins, thresholds set by the trust aligned to national MEWS and NICE NG133, an escalation queue with a response SLA, and GP/community handover.
- **Why different:** it is a service line with a clear budget (NHS virtual-ward funding and maternity safety funding), not a score. It uses validated inputs and guideline thresholds, so it is a decision-support tool under clinical governance rather than a predictive device on day one.
- **Why no one dominates:** general virtual-ward vendors lack maternity workflows; maternity record vendors lack home monitoring; Babyscripts is US-centric.
- **Technical:** device integration (Bluetooth/SDK), offline-first app, rules engine, FHIR interfaces to BadgerNet/K2, SMS fallback.
- **Clinical:** obstetric hypertension pathway owner, clinical safety officer, midwifery workflow design, escalation SLAs, training.
- **Regulatory:** the app displaying readings and rule-based escalation configured by the trust is decision support; classification must still be assessed (likely Class I or "not a device" if no interpretation; TO VALIDATE). BP devices carry their own certification.

- **Data:** BP, urine protein, symptoms, medications, appointments, escalation outcomes.
- **Partners:** one trust with a hypertension clinic; Health Innovation Network; device supplier; Healthy.io; NPEU/Oxford for evaluation design.
- **Revenue:** £40k–£90k pilots; £18–£36 per woman per year plus site licence; ICB-level contracts.
- **Defensibility:** workflow integration, evidence, escalation data, trust relationships.
- **Global:** the same design with cheaper cuffs and SMS in Nigeria and Kenya.

B. The 365-day postpartum companion

- **What:** continuity from discharge to one year: scheduled check-ins on mood (EPDS-aligned prompts), BP after hypertensive pregnancy, bleeding, infection, pain, feeding, contraception, and a structured handover to the GP at 6–8 weeks and beyond; escalation to perinatal mental-health teams by rule.
- **Why different:** maternity care ends at 10–28 days for midwives; the year after is a gap where mental-health deaths cluster. No vendor owns it.
- **Why no one dominates:** the budget is split between maternity, primary care and mental health; a continuity product can be bought by the ICB rather than any one of them.
- **Technical:** app plus SMS/WhatsApp; rule engine; GP record integration (GP Connect, TO VALIDATE); mental-health service referral routes.
- **Clinical:** perinatal psychiatry and health-visiting input; safeguarding design; crisis routing.
- **Regulatory:** screening prompts using validated questionnaires are decision support; careful intended-use wording.
- **Data:** longitudinal postpartum data, largely absent from any dataset today.
- **Partners:** ICB perinatal mental-health leads, health-visiting services, Tommy's, Maternal Mental Health Alliance, a university (King's, Manchester).
- **Revenue:** per woman per year (£10–£20); ICB contracts; employer benefit.
- **Defensibility:** unique dataset; integration across three services.
- **Global:** postpartum follow-up is the weakest link in most systems; SMS-first version travels.

C. Woman-held continuity record with graduated consent

- **What:** a portable maternity record the woman controls, readable by any care setting with her consent (QR, NHS login, or code), synchronising with the provider's system where one exists and standing alone where none does. Consent tiers: care, safety escalation, research, sponsorship.
- **Why different:** records today belong to institutions; women who move, migrate or use private and public care fall between them.
- **Why no one dominates:** interoperability is unglamorous and requires standards work; incumbents are institution-centric.
- **Technical:** FHIR UK Core resources, offline cache, identity verification, audit; integration engine.
- **Clinical:** minimum dataset agreed with RCOG/RCM; safety of partial records.
- **Regulatory:** record-keeping software; data-protection heavy; not a device.
- **Data:** the backbone for everything else.
- **Partners:** NHS England interoperability leads, PRSB (standards), trusts, Nigerian teaching hospitals.
- **Revenue:** platform licence; integration fees; programme contracts.
- **Defensibility:** each interface built raises switching costs; consent architecture is hard to copy.
- **Global:** the primary product for Africa (records that follow women between community and facility).

D. Equity Intelligence for commissioners and ministries

- **What:** anonymised, continuous indicators of reach, engagement, escalation, response time and outcomes by ethnicity, deprivation and geography, with alerts when a group is being under-served; published as reports first, then a product.
- **Why different:** current equity insight is triennial (MBRRACE) and retrospective; this is monthly and actionable.
- **Why no one dominates:** requires patient-level continuity data across settings, which only a continuity layer holds.
- **Technical:** analytics warehouse, small-number suppression, differential privacy where needed.
- **Clinical:** definitions agreed with public-health and maternity leads.
- **Regulatory:** anonymisation standards (ICO code); not a device.
- **Data:** derived from A–C.
- **Partners:** ICBs, NHS Race and Health Observatory, Five X More, LMNS boards, state ministries.
- **Revenue:** commissioner subscriptions (£20k–£60k per ICB per year); report sponsorship.
- **Defensibility:** data and credibility.
- **Global:** the same layer for state programmes and donors who must report equity.

E. The consented longitudinal maternal cohort (data flywheel)

- **What:** women using A–C can opt in to a governed research cohort; multimodal, longitudinal, diverse (UK and Nigeria); access for universities and, under strict rules, industry; the basis for any future predictive model.
- **Why different:** existing cohorts are small, single-country or historic; a diverse digital cohort with postpartum follow-up does not exist.
- **Why no one dominates:** it takes years and trust; it is not a product feature.
- **Technical:** research environment (trusted research environment model), de-identification, federated access.
- **Clinical/ethical:** ethics approvals, participant panels, benefit-sharing.
- **Regulatory:** research governance (HRA), not device law until a model is deployed.

- **Data:** everything above plus optional wearables and voice.
- **Partners:** NPEU, King's, UCL, Liverpool, Born in Bradford, HDR UK, Nigerian universities.
- **Revenue:** research licences, grant-funded studies, pharma real-world evidence, trial recruitment.
- **Defensibility:** the strongest long-term moat in the plan.
- **Global:** by design.

The AI score from the 2025 deck lives inside E, not on the roadmap of A.

PART 3 — REDEFINE THE PRODUCT

A. Ultimate vision (10 years)

The continuity and intelligence layer for pregnancy and the year after birth, used by health systems on three continents: every woman keeps one record that follows her, is never out of contact with a responsible human, is monitored at home with validated devices when her risk warrants it, is supported for a year after birth, and contributes, if she chooses, to a research cohort that makes care safer and fairer for the next woman. Predictive models exist, are validated, regulated and explainable, and are the last thing built rather than the first.

B. Core product (years 1–3)

MaternaLink Continuity: registry and woman-held record; offline-first capture; daily and weekly check-ins; symptom and medication logs; validated home BP and phone urinalysis integration; rule-based escalation aligned to national MEWS and configured by the provider; escalation queue with SLA; notes, exports, integrations; postpartum year; equity analytics; consent tiers; EN/FR plus local languages.

C. First MVP (see Part 13)

A **maternity virtual ward for hypertension** at one trust plus a **community continuity cohort** with one NGO in Nigeria, on the same codebase.

D. 12-month roadmap

Quarter	Deliverables
Q4 2026	Entity, IP, intended-use statement, prototype audit, MVP build start, one trust and one NGO LOI, DPIA, hazard log, Cyber Essentials
Q1 2027	MVP live in service evaluation (UK) and cohort (Nigeria); BP device integration; escalation SLA reporting; first equity report
Q2 2027	Postpartum module v1; BadgerNet/K2 interface scoping; DTAC evidence pack; NICE ESF evidence plan; research cohort protocol to ethics
Q3 2027	Evaluation read-out; second trust; ICB conversation with equity dashboard; grant-funded feasibility study; seed round preparation

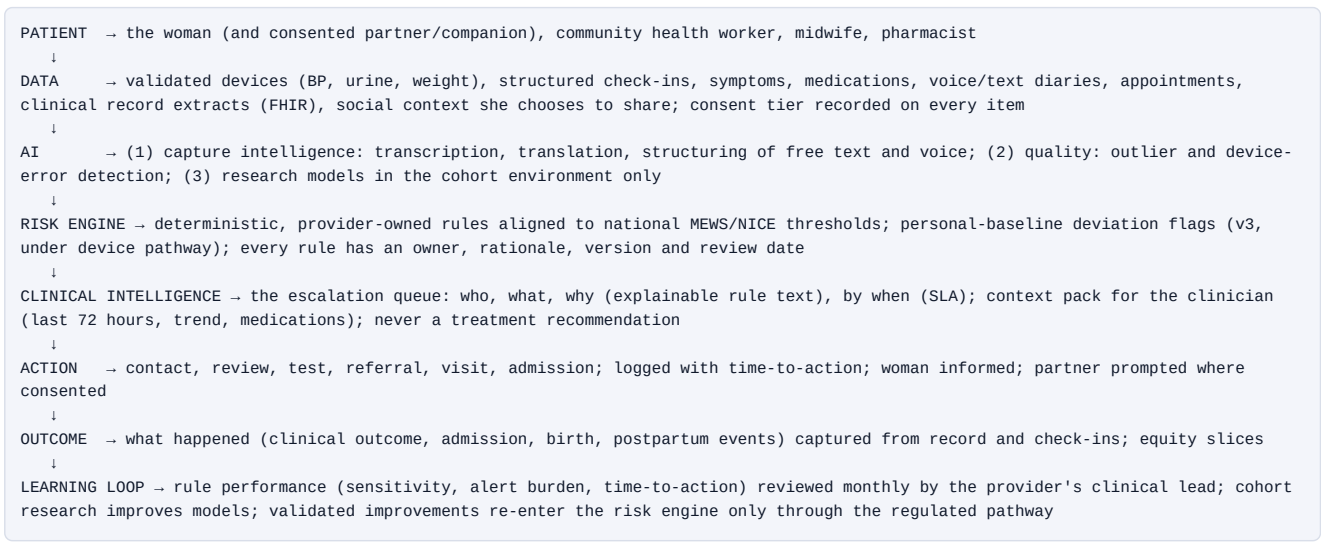
E. 3-year roadmap

Year 1: continuity plus virtual ward, two UK sites, one Nigerian programme, evidence. Year 2: postpartum year, five to eight UK sites, two African programmes, equity product for two ICBs, research cohort open, seed round. Year 3: interoperability at scale, first personalised-baseline decision-support features under a documented device pathway, US or EU partner pilot, Series A.

F. 5-year vision

Fifty UK maternity services, ten programmes across Africa, a research cohort of 250,000+ pregnancies, a regulated predictive module for hypertensive disorders, and equity intelligence used by national bodies.

G. New product architecture



PART 4 — CREATE A NEW CATEGORY

#	Category	Definition	Why large	Products beneath	Hard to replicate because	How we own it
1	Maternal Continuity Infrastructure	The record, channel and escalation layer that keeps every pregnant and postpartum woman connected to care across settings and countries	130m births a year; continuity gaps are universal	Record, check-ins, virtual ward, postpartum, integrations	Interfaces, consent architecture, multi-setting trust	Define the standard, publish the minimum dataset, ship the integrations
2	Perinatal Virtual Care	Home-based, device-supported maternity care as a service line	NHS virtual-ward policy; US remote monitoring reimbursement	Virtual ward, devices, escalation	Clinical workflow depth	Own the hypertension pathway first
3	The Postpartum Year	Care and monitoring from birth to 12 months as its own category	Late maternal deaths; mental health; no incumbent	Postpartum companion, GP handover, mental-health routing	Cross-service integration	Name it, publish the evidence, sell to ICBs
4	Equity Intelligence for Maternity	Continuous measurement of who care reaches and fails	Policy mandate in UK; donor reporting globally	Dashboards, reports, alerts	Needs patient-level continuity data	Publish first, product second
5	Woman-Held Health Records	Patient-controlled, portable records starting with pregnancy	Portability across systems and borders	Record, consent tiers, identity	Standards work, trust	Start with maternity, extend to child and women's health
6	Maternal Safety Operations	The operational layer (queues, SLAs, audits) for maternity safety programmes	Every safety review demands it	Escalation queue, SLA analytics, audit	Workflow lock-in	Sell to safety leads and LMNS
7	Community Maternal Care Platforms	Tools for community health workers and midwives in low-resource settings	Where most births and deaths are	Offline app, USSD/SMS, training	Field-hardened design	Nigeria, Kenya, Ghana first
8	Perinatal Research Infrastructure	Consented digital cohorts and trusted research environments for pregnancy	Pharma, diagnostics and academia lack diverse longitudinal data	Cohort, TRE, recruitment	Years of consent and trust	University consortium
9	Maternal Digital Twin	Personal trajectory models for pregnancy	Long-term research and personalisation	Baseline models, explanations	Data and validation	Research first
10	Life-course Women's Health Continuity	Pregnancy as the entry to a lifelong continuity record (menopause, cardiovascular risk after pre-eclampsia)	Pre-eclampsia doubles later cardiovascular risk; no one follows up	Post-pregnancy cardiovascular follow-up	Longitudinal data	Extend from the postpartum year

Selected category: Maternal Continuity Infrastructure, with Perinatal Virtual Care as the first commercial wedge and The Postpartum Year as the second. It is large, it is not what incumbents sell, it is regulator-safe on day one, and it produces the data that makes every later category (equity, research, prediction) possible.

PART 5 — BUSINESS MODEL

Who should pay (and who should not)

Payer	Pays for	Willingness	Verdict
NHS trusts (maternity, digital budgets)	Virtual ward, continuity licence, integration	Medium; budgets tight; virtual-ward and safety funding help	Primary UK buyer year 1–2
Integrated Care Boards	Postpartum year, equity intelligence, cross-provider continuity	Medium–high for inequality outcomes	Primary UK buyer year 2–3
Private maternity providers and hospitals (UK, Gulf, Nigeria)	Premium continuity and virtual care	High per patient, small volume	Early revenue, good margins
Governments and state programmes (Africa)	Programme deployment, training	Low per woman, high volume, slow	Year 2+, via implementers
Donors and global-health funders	Programme costs, evaluation	High for evidence-generating pilots	Year 1–2
Insurers and employers	Maternity benefit, continuity	Medium (UK), high (US)	Year 2–3
Pharma and diagnostics	Trial recruitment, real-world evidence, adherence programmes	High, but only with a consented cohort	Year 2+
Universities and research funders	Cohort access, studies	Grant-funded	Year 1+
Patients	Nothing essential	Sponsors (diaspora, employers) may pay for others	Never the woman herself for safety features

Five revenue models

- Enterprise platform licence + per-woman fee** (trusts, hospitals, programmes): predictable, scales with births.
- Virtual-ward service fee** per enrolled high-risk woman per month, including device logistics: aligns with NHS virtual-ward economics.
- Commissioner subscription** (ICBs, ministries) for the postpartum year and equity intelligence: cross-provider value.
- Outcome-linked component** (10–25% of fee) tied to auditable process indicators (time-to-contact, postnatal completion, BP follow-up completion): buyer-friendly, evidence-generating.
- Research and evidence licensing**: cohort access, trial recruitment, real-world evidence for pharma and diagnostics; plus **sponsorship** revenue (diaspora and employer per-woman sponsorship in Africa).

Also: integration and implementation fees; training and e-learning; white-label for NGOs and insurers; API access for approved partners.

Strongest model

Enterprise licence + per-woman fee, sold first as a maternity virtual ward to trusts, then as a postpartum and equity subscription to ICBs, with an outcome-linked slice and research licensing layered on from year 2. It matches how the NHS buys, produces evidence as it earns, and creates the dataset that funds the research line.

PART 6 — PARTNER ECOSYSTEM

Tier	Category	Why we need them	What we offer	What we ask	What they get	Approach	Stage	NDA?	Instrument
1 Foundational	One NHS trust maternity service (hypertension pathway)	Without a clinical home there is no product, evidence or credibility	A no-fee or low-fee service evaluation, configured to their pathway, with all safety artefacts	A named clinical lead, a midwifery lead, Caldicott sign-off, 100–300 women, evaluation co-authorship	First-mover recognition, a virtual-ward capability, publication	Via Health Innovation Network and a warm introduction to the obstetric hypertension lead	Now	Mutual NDA before data discussions; not before the first conversation	Service-evaluation agreement + data-sharing agreement
1	One African implementing NGO with a maternal programme	Low-resource validation and the second market	Free platform for a cohort, training, evaluation design	A cohort of 500–2,000 women, community health workers, ethics support	Better continuity data for their funders	Through programme directors and country leads	Now	Yes, mutual	MoU + evaluation protocol
1	Clinical safety officer and regulatory adviser	Nothing ships without them	Fractional retainer, equity in the option pool	Ownership of hazard log, intended use, classification opinion	Founding-team role	Referral via RCM/RCOG networks and Health Innovation Networks	Now	Contract	Consultancy agreement
1	A university perinatal research group	Evidence design, ethics, credibility, the cohort	Data access, co-investigator role, joint grant bids	Evaluation design, ethics sponsorship, PhD/fellow time	Publications, data, grant income	Directly to the professor leading maternal-health digital research	Now	Yes	Research collaboration agreement
2 Technology	Validated BP device makers (Omron, Withings, A&D)	Home readings must be from validated devices	Volume channel, integration showcase	SDK/API access, device pricing, loan stock for pilots	New channel into maternity	Partnerships or medical-channel managers	Month 2	Standard	Supply and integration agreement
2	Smartphone urinalysis (Healthy.io)	Urine protein at home without building CV	Integration in a maternity pathway	API and per-test pricing	Maternity use case	BD lead	Month 3	Standard	Commercial
2	Cloud (AWS, Microsoft Azure, Google Cloud)	Hosting, credits, health-data compliance	A reference health-tech start-up	Start-up credits, architecture review, UK-region compliance support	Case study	Start-up programmes	Month 1	No	Credits agreement
2	Interoperability (FHIR integration engines, NHS APIs; maternity record vendors)	Data must flow to the legal record	Complementary product; joint customers	Interface specs, sandbox, partner listing	Extended value for their customers	Product partnership managers at Clevermed, K2, Magentus	Month 4–6	Yes	Partner/interface agreement
2	Messaging (Twilio, Africa's Talking, Meta WhatsApp Business partners)	SMS/WhatsApp channels	Volume	Pricing, sandbox, compliance support	Volume	Self-serve then account manager	Month 2	No	Commercial
2	Cybersecurity and assurance (Cyber Essentials body, pen-test firm, DSPT consultant)	Assurance evidence	Repeat work	Fixed-price packages	Revenue	Direct	Month 2	Standard	Commercial
2	Voice/speech (transcription and translation APIs; academic speech labs)	Voice capture in many languages	Real-world multilingual maternity use case	On-device or UK-hosted options, language coverage	Data and case study	Direct	Month 6	Yes	Commercial/research

Tier	Category	Why we need them	What we offer	What we ask	What they get	Approach	Stage	NDA?	Instrument
3 Clinical	Additional trusts, private maternity providers, Nigerian teaching hospitals	Scale, diversity, evidence	Service evaluations, fair pricing	Sites, clinicians, data	Capability and evidence	Through the first site's clinicians and networks	Month 6+	Yes	Service evaluation → commercial
3	Royal Colleges (RCOG, RCM), professional networks (digital midwives)	Legitimacy, guideline alignment	Alignment with national MEWS, training content	Review, endorsement of process (not product), events	Member value	Policy and innovation leads	Month 3	No	Engagement, not contract
4 Data	NPEU/MBRRACE-UK, Born in Bradford, ALSPAC, HDR UK, NHS England data services	Validation data, benchmarks, methods	Prospective digital data; co-analysis	Access under governance, methods advice	New data flows	Directors and data-access committees	Month 6–12	Yes	Data access agreements
4	Nigerian data partners (teaching hospitals, DHIS2 programmes)	Low-resource validation	Tools and training	Cohort data under ethics	Capability	Through implementers	Month 6	Yes	Research/data agreement
5 Commercial	Health Innovation Networks, NHS Innovation Service, G-Cloud, procurement frameworks; NGO implementers; telecoms in Africa	Distribution into buyers we cannot reach alone	Evidence, a compliant product	Introductions, listings, bundling	Innovation adoption targets; new services	Innovation exchange leads; partnerships directors	Month 2 (HIN) / Month 9 (frameworks)	No	Listing/reseller agreements
6 Strategic/corporate	Diagnostics and pharma with maternal programmes (Roche Diagnostics, MSD for Mothers, Ferring, Organon), insurers (Bupa, AXA Health), telecoms (MTN, Airtel, Vodafone), device companies (Philips)	Capital, distribution, credibility, adherence and RWE programmes	Access to a consented cohort and pathway	Programme funding, co-development, pilots	Evidence, reach, ESG outcomes	Medical affairs, market access and partnerships leads	Month 9+ (after evidence)	Yes	Co-development / commercial
7 Funding	Angels and syndicates, women's-health and impact VCs, Innovate UK, SBRI Healthcare, NIHR i4i, Wellcome, Grand Challenges Canada, Gates, MSD for Mothers	Capital staged to evidence	A credible plan and data room	Pre-seed, grants, then seed	Returns, impact	See /investors/	Month 3+	No	Investment / grant

PART 7 — PARTNER TARGET LIST (74 organisations)

The full list with all ten fields is in `investors/partners/PARTNER_TARGET_LIST.csv` and the Excel war room. Priorities (score /10) in summary:

UK (priority 8–10): Oxford University Hospitals + Oxford Nuffield Department of Primary Care (home BP research) · Guy's and St Thomas' · King's College Hospital + King's Department of Women & Children's Health · Imperial College Healthcare + Imperial · Birmingham Women's and Children's · Liverpool Women's + University of Liverpool · Manchester University NHS FT (Saint Mary's) + Tommy's Manchester centre · Leeds Teaching Hospitals · UCLH + UCL Institute for Women's Health · NPEU (Oxford; MBRRACE-UK) · Health Innovation Oxford & Thames Valley · Health Innovation Network South London · Health Innovation Manchester · Health Innovation West Midlands · NHS England Maternity and Neonatal programme · NHS Race and Health Observatory · RCOG · RCM · Tommy's · Five X More · Birthrights · Wellbeing of Women · Maternal Mental Health Alliance · Sands · PRSB · HDR UK · Born in Bradford · Clevermed (BadgerNet) · K2 Medical Systems · Magentus (Euroking) · Healthy.io · Omron Healthcare UK · Withings · Bupa · AXA Health · Microsoft UK / AWS / Google Cloud start-up programmes · Vodafone Business.

Europe: Philips (Netherlands) · Erasmus MC (Rotterdam) · Karolinska (Stockholm) · Nina Capital · Heal Capital · EIT Health.

US: March of Dimes · Babyscripts (benchmark, potential partner) · Mayo Clinic Platform · Rock Health · NIH NICHD (data) · Maven Clinic.

Africa and global: mDoc (complementarity partner in Nigeria) · Jacaranda Health · Praekelt (MomConnect) · Lagos State Ministry of Health · NPHCDA · SOGON · MTN Nigeria · Airtel Africa · Safaricom · PATH · Jhpiego · Society for Family Health · UNICEF Nigeria · WHO AFRO · Gates Foundation · MSD for Mothers · Grand Challenges Canada · Fondation Botnar · CIFF.

Decision-maker titles are given per organisation in the CSV (for example: Consultant Obstetrician and Clinical Lead for Hypertension in Pregnancy; Head of Midwifery; Chief Clinical Information Officer; Director of Innovation; Head of Research; Country Director; Head of Medical Affairs; Head of Partnerships).

PART 8 — THE "IMPOSSIBLE PARTNERSHIP" EXERCISE

1. **NHS England (Maternity and Neonatal programme) + NPEU:** national MEWS alignment, equity measurement and evaluation design. Would make MaternaLink the reference continuity layer for the national safety agenda and give access to MBRRACE methodology. Transformational because it removes the "duplicates MEWS" objection and creates policy pull.
2. **Roche Diagnostics (pre-eclampsia PIGF testing):** a joint hypertension pathway where home BP and symptoms decide who is tested, and test results flow into the record. Transformational because it attaches a guideline-backed diagnostic to the pathway, brings market-access muscle and credibility in every NHS trust and export market.
3. **Gates Foundation or MSD for Mothers with a Nigerian state programme and mDoc as the incumbent partner:** funds the low-resource validation at scale, positions MaternaLink as the interoperable continuity layer beneath existing programmes rather than a competitor. Transformational for reach and for the cohort.
4. **Microsoft (UK Health) or Google Health with a university partner:** federated learning infrastructure, speech in African and South Asian languages, credits and go-to-market. Transformational for the AI and voice layer without building infrastructure.
5. **Bupa (UK, plus its international provider network):** the postpartum year and continuity as a covered maternity benefit across the UK, Gulf and other markets. Transformational because it creates a private payer at scale and a bridge to insurers globally.

(Runner-up: a UK telecom such as Vodafone with its African operations, for zero-rated maternity messaging.)

PART 9 — DATA MOAT

Collect (with tiered consent): enrolment and consent events; appointments and attendance; validated device readings with device identity; structured check-ins and symptoms; medications and adherence prompts; escalation events and time-to-action; clinician notes (in the provider's record where one exists); outcomes (from record and check-ins); postpartum year events; language, channel and access data (to measure equity); optional research-tier voice and text diaries; optional wearable data.

Never collect: ethnicity or socioeconomic proxies as predictive features (collect ethnicity only for equity monitoring, with explicit purpose); free-text on partners' conduct without a safeguarding pathway; biometric identifiers beyond what authentication needs; location tracking; data from women who have not consented to the tier in question; anything "because it might be useful later".

How data improves models: rule performance (sensitivity, specificity, alert burden per rule per site) → rule tuning by clinical leads; longitudinal baselines → personal-deviation research; postpartum outcomes → the first credible dataset for late complications; multilingual diaries → speech and NLP research under ethics.

Longitudinal datasets: enrol at booking, follow to 12 months postpartum, link to child immunisation prompts; retain de-identified research copies under the cohort governance; re-contact consent for follow-up studies.

Proprietary datasets: the escalation and response dataset (what was flagged, what happened, how fast) and the postpartum-year dataset do not exist elsewhere; multi-country diversity (UK and Nigeria) is rare.

Avoiding exploitation: benefit-sharing (results returned to participants and communities), a participant panel, no sale of identifiable data ever, industry access only to de-identified, purpose-limited analyses in a trusted research environment, transparent registers of data use.

Privacy protection: data minimisation, pseudonymisation at source, encryption, role-based access, break-glass logging, UK residency for UK data, in-country options for Nigeria, DPIA per deployment, retention schedules, independent audit.

Consent architecture: four tiers (care; safety escalation; research; sponsorship visibility), each separately revocable, recorded on every data item; plain-language and voice consent; partner access controlled by the woman.

Hard to replicate: interfaces to record systems, the escalation-response dataset, the postpartum cohort, and the trust of clinicians and communities, accumulated over years.

The MaternaLink data flywheel: more sites → more continuity data → better rule performance and equity insight → better outcomes and evidence → more sites and commissioners → a larger consented cohort → validated research models → regulated decision support → higher value per woman → more sites.

PART 10 — AI MOAT

Use existing infrastructure for: cloud, transcription and translation base models, OCR for dipsticks (partner), speech-to-text, standard ML tooling, federated-learning frameworks, monitoring.

Make proprietary:

1. **The escalation-response model:** predicting which flags need faster contact given site capacity and history (operational, not diagnostic; lower regulatory burden).
2. **Personal-baseline temporal modelling:** deviation-from-self for BP, symptoms and engagement, with explanations; the candidate for the first regulated module.
3. **Multilingual maternity language models** fine-tuned on consented diaries for structuring, not diagnosis.
4. **Fairness monitoring:** subgroup performance dashboards, drift detection, alert-burden equity.
5. **Clinical reasoning layer:** rule-text explanations, context packs, and audit of clinician actions to learn which flags mattered.
6. **Continual and federated learning pipelines** across sites and countries with governance gates; models retrained only through documented change control.
7. **Human-in-the-loop by design:** every model output is a prompt to a human with a recorded decision; that decision is training signal.

Rules for the moat: models never ship without subgroup validation; explainability is a product feature; the AI roadmap follows the data flywheel, not the pitch deck.

PART 11 — REGULATORY REALITY CHECK (UK)

Stage	What it is	Governance	Timing (ESTIMATE)
IDEA → PROTOTYPE	Continuity platform; rule-based escalation configured by the provider; no interpretation	Intended-use statement; DPIA; DCB0129 clinical safety case; Cyber Essentials; DSPT; hazard log; classification assessment documented (likely not a device or Class I as record-keeping and provider-configured rules; TO VALIDATE with MHRA guidance)	Months 0–4
RESEARCH	Cohort studies, model development in a research environment	HRA/REC ethics approval via IRAS; research governance; no clinical deployment of models	Months 4–24
PILOT	Service evaluation at a trust (not research), then feasibility studies	Trust governance, Caldicott, DTAC evidence pack, NICE ESF Tier A/B evidence plan, evaluation registered	Months 4–12
VALIDATION	Prospective validation of any predictive module across subgroups	Clinical investigation under MHRA rules if the module is a device; ISO 14155-style conduct	Months 18–36
REGULATORY	UKCA marking of the predictive module as SaMD (likely Class IIa or higher under the forthcoming UK regime aligned with EU MDR Rule 11; TO VALIDATE); QMS ISO 13485; post-market surveillance; MHRA AI Airlock sandbox as an option	Notified/approved body engagement	Months 24–40
NHS DEPLOYMENT	Procurement via framework; DTAC pass; clinical safety case DCB0160 by the deploying trust; AI governance (explainability, human oversight, bias monitoring, incident reporting)	Ongoing	Month 12 (continuity) / Month 36+ (predictive module)

Distinguish clearly: *Research tool* (cohort environment; no patient-facing outputs) · *Decision-support tool* (displays data, applies provider-owned rules, explains; may still be a device depending on claims; keep claims to workflow) · *Clinical software* (record-keeping, communication; data-protection and clinical-safety governed) · *Medical device/SaMD* (any output that predicts, diagnoses or recommends for an individual; full pathway).

PART 12 — CLINICAL VALIDATION

Study 1 (year 1): a service evaluation, then a feasibility study, of the Maternity Virtual Ward for hypertensive disorders.

- **Participants:** pregnant women ≥ 20 weeks with chronic hypertension, gestational hypertension, or high risk of pre-eclampsia per NICE NG133, plus postpartum women with hypertensive disorders up to 6 weeks; at one trust; community and clinic settings.
- **Sample size:** service evaluation 100–150 women (descriptive); feasibility study 200–300 to estimate recruitment, adherence, escalation rates and time-to-action with adequate precision; a later effectiveness trial would need ~1,500–3,000 women for process outcomes and far more for rare clinical outcomes (statistician to confirm).
- **Inclusion/exclusion:** able to consent; smartphone or SMS access (or supported alternative); excludes women already under inpatient care; language support provided rather than exclusion.
- **Data:** home BP (validated cuff), urine protein (phone dipstick or clinic), symptoms, medications, appointments, escalations, actions, admissions, birth outcomes, satisfaction, midwife time.
- **Primary endpoint (feasibility):** proportion of clinically significant readings (above trust thresholds) with documented clinician contact within the agreed SLA (e.g., 4 hours).
- **Secondary:** adherence to monitoring schedule; time from reading to contact; unplanned attendances; admissions; antenatal appointments saved; women's and midwives' experience; equity of reach and response by ethnicity and deprivation; safety events; alert burden per midwife.
- **Bias analysis:** subgroup reporting by ethnicity, deprivation, language, age, BMI; missingness analysis; device access.
- **Safety monitoring:** hazard log, incident reporting, clinical safety officer review weekly, stopping rules.
- **Comparator:** usual care (historical controls for the evaluation; concurrent control site or stepped-wedge for the feasibility study).
- **Success:** $\geq 80\%$ adherence; $\geq 90\%$ of significant readings contacted within SLA; no safety signal; equitable reach; midwives willing to continue; a publishable evaluation; a trust that wants to renew.

What makes a trust take you seriously: a clinical safety case, a DTAC evidence pack, a named clinician co-author, an evaluation protocol reviewed by a university, subgroup data, and a price that maps to a budget they already have.

PART 13 — MVP (limited funding)

The smartest first product: the **Maternity Virtual Ward for hypertension** at one UK trust, sharing a codebase with a **community continuity cohort** in Nigeria. It is buildable in 16–20 weeks, produces evidence a trust and an investor both value, creates the escalation-response dataset, and is the wedge into the continuity platform.

Item	Specification
Features	Enrolment and consent tiers; woman app (PWA) and SMS fallback; validated BP cuff pairing (Bluetooth) with manual entry; phone dipstick via partner API (or manual entry in MVP); symptom and medication check-ins; provider-configured thresholds aligned to national MEWS/NICE; escalation queue with SLA timers and contact logging; timeline and notes; exports; equity report; audit log; EN plus one local language
Tech stack	TypeScript/Next.js PWA; Node (NestJS) API; PostgreSQL; Redis; object storage; Auth0 or Keycloak; AWS eu-west-2 (UK); Twilio/Africa's Talking; FHIR export; Terraform; observability; automated tests; no patient data to third-party AI
Team	Founder (product/commercial), fractional CTO/lead engineer, one full-stack engineer, one mobile/front-end engineer (contract), clinical safety officer (fractional), midwifery adviser (sessional), designer (contract), evaluation lead (university partner)
Timeline	Weeks 1–4 discovery, intended use, DPIA, hazard log, design; weeks 5–14 build; weeks 15–18 testing, Cyber Essentials, safety case, training; weeks 19–20 go-live; 12-week evaluation thereafter
Cost range (ESTIMATE)	£140k–£260k to first evaluation read-out (UK-led build), or £95k–£180k with a blended UK/Nigeria team; devices £8k–£15k for 100 women; assurance £10k–£20k
Partners	One trust; Health Innovation Network; university evaluation partner; BP device supplier; Healthy.io (optional in MVP); one Nigerian NGO; cloud credits
Pilot	100–150 women over 12 weeks at the trust; 500 women in the Nigerian cohort; weekly service reviews; evaluation report at week 16
Success metrics	≥ 80% monitoring adherence; ≥ 90% significant readings contacted within SLA; median time-to-contact under 2 hours; zero unmitigated safety incidents; equitable reach; NPS ≥ 40 among midwives; trust renewal intent; investor-ready evidence pack

PART 14 — PARTNER PITCH

One sentence: "We keep every pregnant and postpartum woman connected to a responsible clinician between appointments, with home readings and rules your own team sets, and we measure who care reaches and who it misses."

30 seconds: "Most maternal harm happens between appointments, and after discharge, where today's systems are blind. MaternaLink is a continuity layer: home blood-pressure and symptom monitoring, escalation rules your clinicians own, a queue with a response time, and a record that follows the woman for a year after birth. It works offline and on any phone, and it reports equity of reach by ethnicity and deprivation. We're inviting one maternity service to co-design and evaluate the first hypertension virtual ward with us, with a university evaluation partner and the safety artefacts done properly."

2 minutes: add the honest status (pre-seed, prototype under audit, no claims), the evidence plan (service evaluation → feasibility → validation), what you ask (a clinical lead, 100–150 women, 12 weeks), what they get (a working virtual ward, co-authorship, first-mover access, no licence fee for the evaluation), why now (national MEWS, virtual-ward policy, inequality mandate, MBRRACE findings), why you (Vytalix's discipline: intended use, hazard log, DPIA before code; UK–Africa design), what it could become (the continuity infrastructure for maternity, with research and equity intelligence built on top).

Partnership email (trust clinical lead): Subject: Co-designing a hypertension virtual ward for [Trust] maternity, 12-week evaluation. Dear Dr [Name], I'm the founder of Vytalix, a UK health-technology group. We are building MaternaLink, a continuity and home-monitoring layer for maternity. Rather than sell you software, I'd like to co-design and evaluate a hypertension virtual ward with your team: validated home BP, symptom check-ins, escalation thresholds you set, a queue with a response time, and equity reporting, evaluated with [university] and with the clinical safety case and DPIA done first. No licence fee for the evaluation; we ask for a clinical lead, midwifery time and 100–150 women. Could we have 20 minutes? Honest status: we are pre-seed and have not yet deployed in the NHS. [Signature]

LinkedIn (under 300 characters): "Dr [Name], I'm building a maternity continuity and home-monitoring layer (home BP, symptom check-ins, escalation rules your team owns, equity reporting). Looking for one clinical lead to co-design a 12-week evaluation. May I send a one-pager?"

WhatsApp (warm introductions only, never cold): "Hi [Name], [Introducer] suggested I reach out. I'm building MaternaLink, a continuity layer for maternity: home BP monitoring plus escalation rules the team owns, and postpartum follow-up. We're looking for a first evaluation site. Could I send a one-page summary and find 20 minutes next week?"

Meeting opening: "Thank you for the time. I'd like to leave with two things: your view on whether the between-appointment gap is where you lose women, and whether an evaluation like this could fit your pathway. I'll be honest about where we are, and I'd rather hear your objections now than in six months."

Proposal structure: 1 Context and the gap (their data, their words) · 2 What we propose (pathway, features, thresholds owned by them) · 3 Safety and governance (intended use, hazard log, DPIA, DTAC plan) · 4 Evaluation design and co-authorship · 5 What we ask · 6 What they get · 7 Timeline · 8 Costs (evaluation fee or none; device costs) · 9 What happens after (commercial terms, no obligation) · 10 Appendices (security, data flows, consent materials).

Why us / why now / why you / why it matters / what we want / what you get / what it could become are answered in that order in every pitch.

PART 15 — PARTNER OFFER (founding packages)

Package	Role	Contribution	Benefits and recognition	Access	Commercial	Equity?	Exclusivity
Founding Clinical Partner (1–3 UK sites, 1–2 African)	Co-design and evaluate	Clinical lead, midwives, women, governance	Named founding partner; co-authorship; input into roadmap	Early features; steering group seat	No licence fee during evaluation; 40% discount for 3 years after	No equity to institutions; individual clinicians may join the advisory board with small option grants	None; "founding" is a title, not exclusivity
Founding Technology Partner (device, cloud, messaging, urinalysis)	Provide integrations and credits	SDKs, devices, credits, engineering support	Reference partner; joint case studies	Roadmap input	Preferred-supplier status at fair market prices	No	Category exclusivity for 12 months only if they fund it
Founding Research Partner (1–2 universities)	Evaluation and cohort	Design, ethics, analysis, grant bids	Co-authorship; data access under governance	Cohort governance seat	Grant-funded; no fees to Vytalix	No; joint IP on research outputs defined in the agreement	None
Founding Data Partner (cohorts, HDR UK, trusts)	Data access and linkage	Governed datasets, methods	Acknowledgement; joint publications	Research environment	Data-access fees to them, not from them	No	None
Founding Commercial Partner (implementers, HINs, insurers, telecoms)	Distribution	Introductions, bundling, channel	Revenue share; joint marketing	Partner portal	Referral or reseller margin 10–25%	No	Territory exclusivity only with volume commitments
Founding Strategic Partner (diagnostics, pharma, corporate)	Co-development and funding	Programme funding, market access	Board observer if investing; joint pathways	Cohort access under governance	Co-development fees; RWE licences	Equity only for cash investment at market terms (strategic round), never for "help"	Field-limited, time-limited, never global

Rules: equity is exchanged for cash, for a founding executive's time, or for a small advisory grant (0.1–0.5% each, vesting); everything else is paid or commercial. No institution receives equity. Exclusivity is sold, not given, and always narrow and time-boxed.

PART 16 — BRAND

Assessment of "MATERNA-LINK": descriptive, warm, and clear in English; the hyphen is untidy and the prototype uses three spellings (Materna-Link, MaternalLink, Maternal Link); "-Link" is common (dozens of "-Link" health products) and hard to trademark distinctively; it does not travel into French/Yoruba/Hausa contexts badly, but it says "app", not "infrastructure". Also a search must check "Maternalink"/"MaternaLink" conflicts in classes 9, 42, 44 (TO VALIDATE).

Recommendation: keep **Vytalix** as the parent (strong, distinctive, already designed); keep **MaternaLink** (one word, no hyphen) as the product family name for now because it has recognition with your existing contacts; plan a product-family architecture: *MaternaLink Continuity* (record and check-ins), *MaternaLink Home* (virtual ward), *MaternaLink Year* (postpartum), *MaternaLink Equity* (commissioner intelligence), *MaternaLink Research* (cohort). Re-evaluate the master brand at Series A with a professional naming study if the category positioning needs a less descriptive name.

Twenty alternatives (all TO VALIDATE for trademark and domain): Contina · Amara Health · Nurta · Velia · Kindra · Orla Health · Lumea · Tessera · Continuum Maternity · Harbour · Bridgewell · Amaryllis · Solace Health · Kaira · Vita Materna · Everline · Tenda · Cradle Continuity · Wellspring Maternal · Halo Maternity. Shortlist for a category-defining brand: **Contina** (continuity; short; distinctive), **Nurta** (nurture; global), **Velia**, **Tessera** (pieces joined into one record). None should be adopted without clearance.

PART 17 — GLOBAL SCALE

Market	What changes	Sequence
UK	Trust and ICB buyers; DTAC/MEWS alignment; NHS virtual-ward economics; equity mandate	Years 1–3
Europe	EU MDR for any predictive module; GDPR; national procurement; Netherlands, Nordics and Ireland as first markets via partners (Philips, Erasmus MC, Karolinska)	Year 3+
North America	Reimbursement for remote monitoring (CPT codes) and Medicaid maternal programmes; Babyscripts as incumbent; partner-led entry via a health system or insurer	Year 3–4
Africa	Programme and donor buyers; SMS/USSD/WhatsApp-first; community health-worker mode; in-country hosting; DHIS2 alignment; sponsorship model	Year 1 (Nigeria) → Year 2–3 (Kenya, Ghana, Rwanda)
Middle East	Private hospital groups and national digital-health programmes; premium virtual care; Arabic	Year 3+
Asia-Pacific	India and Bangladesh via NGO and government programmes; ASHA-type workforce mode	Year 4+

Low-resource design: the same continuity record on a feature phone through USSD and SMS; offline tablets for community health workers; cheap validated cuffs shared at community posts; paper-to-digital bridging; local-language voice; in-country data residency; results-based and sponsorship funding. **Global infrastructure layer:** achievable if the record and consent architecture are standards-based, the escalation engine is provider-owned, and every market adds interfaces rather than forks.

PART 18 — THE £1 BILLION TEST

Honest answer: possible but improbable on maternity software alone; plausible only as maternal continuity infrastructure with research and multi-market revenue. UK maternity software at £30 per birth is an £18m annual market. A £1bn valuation needs roughly £100m+ recurring revenue at a 8–10× multiple, or a data and research franchise that commands more. That requires millions of women a year across several continents, plus research and pharma revenue, plus a regulated predictive module. The realistic strong outcome is a £50–150m company; the £1bn path exists and is set out below so the choices that keep it open are made early.

Stage	Product	Customers	Revenue	Team	Data	Partnerships	Geography	Capital	Valuation logic
£0 → £1M	Continuity + virtual ward; advisory services	3–5 UK sites; 1–2 African programmes; advisory clients	£1m ARR-equivalent (mix of services, pilots, licences)	6–8	5,000 pregnancies	HIN, university, device, NGO	UK, Nigeria	Pre-seed £0.4m + grants	Pre-seed/seed 3–5× forward
£1M → £10M	Postpartum year; equity intelligence; integrations	25 UK sites; 3 ICBs; 3 programmes; first insurer	£10m	35	100,000 pregnancies; cohort open	Diagnostics co-development; telecom	UK, Nigeria, Kenya, Ghana	Seed £2–3m, Series A £8–12m	6–10× ARR
£10M → £50M	Regulated predictive module; research licensing; sponsorship at scale	100 UK sites; 10 ICBs; 10 programmes; 2 US health systems via partner; 2 insurers	£50m	150	1m pregnancies	Pharma RWE; national programmes	+ EU, US, Gulf	Series B £30–50m	8–12× with data premium
£50M → £100M	Life-course women's-health continuity; federated network	National contracts; payers	£100m	300	3m+	Government-scale	6+ countries	Series C or profitability	8–10×
£100M → £1B	Global maternal continuity infrastructure standard	Multi-national systems, payers, pharma	£120–150m+ with 25%+ margins	400+	Largest consented perinatal cohort	Standards bodies, global funders	Global	Growth equity / IPO	8–10× revenue plus strategic premium

What must change for this path: the concept must move from "AI score" to "infrastructure"; the company must own consent and interoperability; research revenue must be designed in from year 2; a US or insurer partner is needed by year 3; and the founder must build a team with a clinician, an ML lead and a commercial lead who have done this before.

PART 19 — 90-DAY EXECUTION PLAN (partner-first)

Weeks 1–2

- Legal blockers: name clearance; incorporate; IP heads of terms with David Agunede; retire the 2025 deck; prototype behind auth with synthetic data. (Do not pitch anyone until the deck is retired.)
- Write the one-page **intended-use statement** and the **virtual-ward evaluation concept** (2 pages) using Part 12.
- Contact: the **Innovation Exchange lead at your regional Health Innovation Network** (ask: "Which two trusts in the region have active hypertension-in-pregnancy remote-monitoring interest or a digital midwife?"); the **Head of Research at Tommy's** (ask: "Would you review an evaluation design and connect us to a clinical champion?"); **Five X More co-founders** (ask: "Would you be a community partner on the equity design?"); one **professor leading maternal-health digital or hypertension research** (Oxford primary-care BP group; King's Women & Children's Health) (ask: "Would you be the evaluation partner and ethics sponsor for a feasibility study?").
- Build: nothing new; audit the prototype; set up the partner war room (Part 20).
- Do not: build AI models; attend generic start-up events; cold-message 50 people.

Weeks 3–4

- Meetings: two trust conversations (Consultant Obstetrician hypertension lead + Head of Midwifery/Digital Midwife) via HIN introductions; one university; one device supplier (Omron/Withings medical channel manager) for loan stock.
- Documents: clinical safety plan (DCB0129 outline), DPIA template, partner proposal (Part 14 structure), one-pager, data-room v0.
- Evidence: 10 discovery interviews with midwives and obstetricians (questions: "What happened the last time a woman's BP rose between appointments?" "How do you find out?" "Who calls her back and how fast?" "What would make home readings safe for you?").
- Nigeria: reply to Ekiti with a complementarity proposal; approach one implementer (Society for Family Health, Jhpiego Nigeria) about a 500-woman continuity cohort.

Month 2

- Secure: one trust LOI for a service evaluation; one university partner letter; one NGO MoU draft; cloud credits; device loan agreement.
- Build: MVP sprint 1–4 (enrolment, check-ins, BP entry, thresholds, queue) with the fractional CTO.
- Investor prep: update one-pager and deck with the reframe; SEIS/EIS advance assurance submitted; Call List 1 verified.
- Grants: Innovate UK Smart or SBRI Healthcare application outline with the trust's letter.

Month 3

- Sign: service-evaluation agreement and data-sharing agreement; Cyber Essentials; pen-test booked.
- Build: sprints 5–8; safety case v1; training materials.
- Partners: Roche Diagnostics medical affairs conversation (pathway alignment, not money); NHS England maternity programme contact via the HIN; Bupa maternity lead exploratory call.
- Investors: first 10 calls (angels, GC Angels/NorthInvest-type syndicates, women's-health funds) only after the LOI exists.
- Evidence: interview synthesis; first equity report design.

PART 20 — THE PARTNER WAR ROOM

CRM columns: Organisation · Country · Category · Partner Tier · Decision Maker · Job Title · LinkedIn · Email · Phone · Relationship Status · Priority (/10) · Strategic Value (/10) · What We Want · What We Offer · Last Contact · Next Action · Meeting Date · Partnership Stage · Probability (%) · Potential Value (£) · Notes · Source · Owner · NDA status · Instrument (LOI/MoU/SEA/DSA/commercial) · Consent to contact (GDPR basis).

Pipeline: TARGET → RESEARCHED → CONTACTED → RESPONSE → MEETING → DISCOVERY → PROPOSAL → PILOT → MOU → PARTNERSHIP → SCALE. Rules: RESEARCHED requires a named title and a reason; CONTACTED requires a personalised message; PROPOSAL requires the Part 14 structure; PILOT requires safety artefacts; MOU/agreements go through a solicitor.

KPIs (monthly): targets researched (≥ 30); contacted (≥ 20); response rate (≥ 30%); meetings (≥ 8); discovery-to-proposal conversion (≥ 40%); proposals out (≥ 3); pilots/LOIs signed (1 in month 2, 2 by month 3); time from first contact to meeting (≤ 14 days); partner NPS after meetings; pipeline value weighted by probability; number of Tier-1 partners secured (target 4 by Day 90: trust, university, NGO, CSO).

The Excel version ([investors/partners/PARTNER_WAR_ROOM.xlsx](#)) contains the CRM, the 74-organisation target list pre-loaded at stage TARGET/RESEARCHED, the pipeline definitions and a KPI sheet.

FINAL OUTPUT

THE NEW MATERNA-LINK

MaternaLink should become the maternal continuity infrastructure for health systems: a woman-held record, a channel that never goes silent, validated home monitoring when risk warrants it, escalation rules owned by the clinicians who carry the liability, a year of postpartum follow-up, and equity intelligence that shows who care reaches and who it misses, all built to work offline on any phone in Birmingham and in Ekiti alike. The predictive AI from the 2025 deck becomes the research programme that sits on top of a consented cohort, validated and regulated before it ever touches a clinical decision. Vytalix is the parent; MaternaLink is the family of products; the first product is a hypertension virtual ward evaluated properly at one trust.

THE BIG IDEA

Close the 99% of pregnancy and the year after birth that maternity systems cannot see, by keeping every woman connected to a responsible clinician with a record that follows her.

THE CATEGORY

Maternal Continuity Infrastructure, entered through Perinatal Virtual Care and The Postpartum Year.

THE MOAT

The escalation-and-response and postpartum datasets no one else holds, the interfaces and consent architecture that make switching costly, and the clinical trust earned by never over-claiming.

THE FIRST PRODUCT

A hypertension-in-pregnancy virtual ward: validated home BP, phone urinalysis, symptom check-ins, provider-owned thresholds aligned to national MEWS, an escalation queue with a response time, and equity reporting.

THE FIRST CUSTOMER

One NHS trust maternity service with an obstetric hypertension lead and a digital midwife, bought as a 12-week service evaluation that converts to a licence.

THE FIRST PARTNER

The regional Health Innovation Network's Innovation Exchange lead, who opens the trust door, followed within the same fortnight by a university perinatal research group as evaluation partner.

THE FIRST £1M

Roughly: £250k–£400k from Vytalix Advisory and Consulting (investment-readiness, DTAC-readiness and Africa market-entry sprints for health-tech companies and NGOs) in the first 12–15 months; £150k–£250k from two or three paid pilots and first-year licences after the free evaluation (UK trusts at £40k–£90k pilots converting to £100k+ licences); £150k–£300k from one African programme or NGO contract via an implementer; £100k–£250k of grant income (Innovate UK, SBRI Healthcare) if won; and £50k–£100k from early sponsorship and training. That is the first £1m: services first, pilots second, programme third, grants alongside.

THE 10 MOST IMPORTANT PARTNERS (ranked)

1. Regional Health Innovation Network (door-opener).
2. One NHS trust with a hypertension pathway (first customer).
3. A university perinatal research group (Oxford NPEU/primary-care BP group or King's Women & Children's Health) (evidence).
4. A fractional clinical safety officer and regulatory adviser (licence to operate).
5. Tommy's and Five X More (community legitimacy and equity design).
6. A validated BP device maker (Omron/Withings/A&D) (inputs).
7. A Nigerian implementer (Society for Family Health, Jhpiego Nigeria or mDoc as complementarity partner) (second market).
8. Maternity record vendor (Clevermed/K2) (interoperability).
9. Roche Diagnostics medical affairs (pathway credibility, later co-development).
10. A lead angel syndicate or women's-health fund (capital).

THE 10 MOST IMPORTANT ACTIONS (ranked)

1. Sign IP heads of terms with David Agunede.
2. Clear the name and incorporate.
3. Retire the 2025 deck and put the prototype behind authentication.
4. Write and get clinical sign-off on the intended-use statement.
5. Book the Health Innovation Network conversation.
6. Approach one university evaluation partner.
7. Recruit the clinical safety officer.
8. Run 10 discovery interviews with the four questions above.
9. Secure a trust LOI for the hypertension virtual-ward evaluation.
10. Submit SEIS/EIS advance assurance and prepare Call List 1.

THE BIGGEST RISK

That you keep pitching the AI score. It is the part of the concept that is scientifically unsupported, regulatorily heaviest and commercially weakest, and every clinician, regulator and serious investor will see it. The second risk is doing everything alone: without a clinician and an engineer beside you, the partner network will not believe the plan.

THE BIGGEST OPPORTUNITY

To own the continuity of maternity care between appointments and after birth, in the UK and across Africa, as the infrastructure other systems plug into, and to hold the most diverse consented perinatal dataset in the world within five years, which is what would make a validated predictive model, and a category-defining company, possible.

MY NEXT 72 HOURS

Day 1 (morning): message David Agunede to fix a call this week; draft IP heads of terms from 02_GROUP_STRUCTURE.md ; run trademark knock-out searches for Vytalix and MaternaLink; stop circulating the 2025 deck. **Day 1 (afternoon):** write the one-page intended-use statement (draft in product/MATERNALINK_FEATURE_ADDENDUM.md); ask David to put the prototype behind a login and remove photos. **Day 2 (morning):** find your regional Health Innovation Network's Innovation Exchange contact and send the Part 14 email adapted to them; send LinkedIn notes to Tommy's Head of Research and to the Five X More co-founders. **Day 2 (afternoon):** identify the professor leading home BP-in-pregnancy research (Oxford) and the King's Women & Children's Health digital lead; send the research-partner email; open the partner war room and enter the 74 targets with owners and next actions. **Day 3 (morning):** call two device makers' medical-channel teams (Omron, Withings) for loan stock and SDK access; email one Nigerian implementer with the complementarity proposal. **Day 3 (afternoon):** rewrite the one-pager with the continuity framing; list ten midwives and obstetricians for discovery interviews and send the first five requests; book the incorporation with your accountant for the day the name clears.

**BUSINESS ASSETS**

VYTALIX — Investor one-pager

Vytalix company-build documentation · reference

INVESTOR_ONE_PAGER

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

DATE

8 September 2026

VERSION

1.0 · Confidential draft

Contents

What Vytalix is	1
The problem	2
What we are building (PROPOSED)	3
Where we are (honest)	4
Business model (ESTIMATE)	5
The raise (PROPOSED)	6
Why now, why us	7
Ask	8

Technology for Life · UK health-technology group · Pre-seed · [Founder name] · [email] · [phone] · [website]

Status labels: everything below is KNOWN unless marked ESTIMATE, PROPOSED or TO VALIDATE. Nothing here is a claim of traction we do not have.

What Vytalix is

A UK health-technology group with four pillars: **Advisory** and **Consulting** (founder-led services for healthcare organisations, health-tech companies, NGOs and governments), **Health Solutions** (first product **MaternaLink**, a maternal care-coordination and communication platform, in development) and **E-Learning** (planned, year 2). Services fund the company; MaternaLink is the venture story.

The problem

- UK maternity reviews (Ockenden, Kirkup) repeatedly identify communication and coordination failures; MBRRACE-UK reports persistent inequalities in maternal outcomes for Black and Asian women.
- In Nigeria, maternal mortality remains among the highest globally, and continuity of contact between women and facilities is weak, especially outside cities.
- Existing tools are either clinical record systems, consumer apps, or donor-funded programmes; few sit in the coordination layer between women, midwives and facilities in a way that is safe, low-bandwidth and interoperable.

What we are building (PROPOSED)

MaternaLink v1: non-diagnostic, offline-tolerant coordination for antenatal to postnatal care: appointment and contact continuity, two-way messaging with escalation to humans, education content governed by clinicians, and programme dashboards. Designed to stay outside medical-device classification, with a documented intended-use statement, clinical-safety governance and UK GDPR compliance. A clinical-AI risk-scoring concept exists as a **research track** only, gated behind data partnerships, ethics approval and a regulatory pathway.

Where we are (honest)

Have	Building	Next
Founder brief and full company build (strategy, product, finance, brand, website, 300-investor CRM)	Entity incorporation, name clearance, IP agreement with the MaternaLink collaborator (Days 1–14)	First paid advisory engagement (target Day 45)
A prototype dashboard (under audit) and a prior government conversation in Nigeria (Ekiti State)	Advisory board recruitment: obstetric/midwifery, NHS digital, Nigerian health system, regulatory	One UK service-evaluation site and one Nigeria NGO cohort in design (targets Day 90)
Re-based product strategy, PRD, MVP spec, roadmap	Research report: "The State of Digital Maternal Care Coordination — UK and Nigeria" (Week 9)	SEIS/EIS advance assurance (applied for after incorporation; TO VALIDATE)

Business model (ESTIMATE)

Advisory day rates £1,200–£1,800; consulting sprints £15k–£60k; MaternaLink pilots £40k–£150k then per-woman-per-year licences (UK £8–£40; Africa £3–£12 at scale). Three-year Base scenario revenue £1.47m with services 59% of the total; break-even not reached within 36 months in Base; see [/docs/08_FINANCIAL_MODEL.md](#).

The raise (PROPOSED)

Pre-seed £250k–£500k (SEIS/EIS-eligible, TO VALIDATE) to fund a product lead, clinical-safety artefacts, the MVP build and two pilots. Milestones before raising: entity, IP deed, retired clinical claims, first services revenue, one pilot LOI. Later rounds only on evidence: Seed £1.5m–£3m (2028), Series A a 2029–30 decision.

Why now, why us

Policy attention on maternity safety and inequality in the UK; state-level digital-health programmes in Nigeria; low-cost messaging infrastructure; a founder who is building the company in the open with a truth-first operating standard and a disciplined investor process.

Ask

20 minutes to test fit against your mandate. Data room available on request once the entity and IP deed are in place.

**STRATEGY & OPERATIONS**

Maternalink Pricing: Per Product And Annual Service

Vytalix company-build documentation · reference
21_MATERNALINK_PRICING

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

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Contents

Executive view	1
1. Product catalogue (what is sold)	2
2. Cost-to-serve (why the prices are what they are)	3
3. Price book A — United Kingdom (ESTIMATE)	4
4. Price book B — Africa and programme funders (ESTIMATE)	5
5. Commercial rules	6
6. The Maternity Watch: build, buy or partner	7
7. Validation plan (before any price is quoted externally)	8
Priorities / Risks / Next actions	9

Owner: CFO with Chief Product Officer · **Status:** PROPOSED v1.0 (8 September 2026) · **Every figure is an ESTIMATE until validated with three buyers per segment** · Supersedes the £300/woman/year figure in the March 2026 Ekiti proposal, which is retired. Governing files: [/FACTS_BASE.md](#) , [/docs/01_BUSINESS_MODEL.md](#) , [/docs/08_FINANCIAL_MODEL.md](#) .

Executive view

1. MaternaLink is priced as **a platform licence plus optional modules, plus an annual service plan**, with hardware (the Maternity Watch) priced separately as a per-device lease or purchase. This separates software value from device cost and lets buyers start without hardware.
2. Two price books: **UK** (NHS trusts and ICBs, private maternity, employers) and **Africa / programme** (state ministries, NGOs, insurers), because willingness to pay and cost-to-serve differ by an order of magnitude.
3. Indicative annual software price per woman enrolled: **UK £18–£36; Africa programme £4–£10** at scale, with a pilot minimum so no contract is below cost. The old £300/woman/year figure is 8–30× these levels and is not defensible against comparable programmes.
4. Annual service is priced at **18–22% of licence value** (Standard) rising to 30–35% (Premium, 24/7 clinical-safety on-call and named account management), in line with enterprise health-software norms (ESTIMATE).
5. Gross margin target: software 75–85%; services 45–55%; hardware 20–30% (hardware is a channel, not a profit centre).

1. Product catalogue (what is sold)

Code	Product	What the buyer gets	Unit	Regulatory posture
ML-CORE	MaternaLink Core platform	Patient registry (mother + newborn), offline-first capture of symptoms and vitals, timeline, clinician notes, escalation queue, supervisor dashboards, EN/FR interface, exports, audit log	Per site licence + per enrolled woman	Non-diagnostic care-coordination software; intended-use statement controls scope
ML-TRIAGE	Maternity Triage module	Structured triage intake for maternity assessment units, prioritisation rules configured by the site's clinical governance, contact and outcome logging, waiting-time analytics	Per site	Rules configured and owned by the provider; Vytalix supplies the rule editor, not the rules. Classification TO VALIDATE with MHRA guidance before any rule ships pre-configured
ML-ENGAGE	Woman & partner engagement module	Daily questionnaire, symptom log (including itching of hands and feet, vomiting, headache, visual disturbance, reduced movements), education content signed off by clinicians, partner quizzes and prompts, appointment reminders via SMS/WhatsApp	Per enrolled woman	Non-diagnostic; symptom logs route to a human. No automated diagnosis
ML-GLUCOSE	Diabetes-in-pregnancy tracking	Self-reported blood-glucose entries with target ranges set by the clinician, trend view, threshold notifications to the care team	Per enrolled woman with the condition	Threshold notifications configured by the clinician; classification TO VALIDATE (likely needs a device pathway if the software interprets readings)

Code	Product	What the buyer gets	Unit	Regulatory posture
ML-VOICE	Voice diaries	Recorded diaries and voice notes attached to the timeline; transcription and translation as a roadmap option	Per enrolled woman	Non-diagnostic storage and playback only. "Voice tracing of baby movements" is a research track , not a sold feature, until validated
ML-WATCH	Maternity Watch (device)	A wearable issued at the 16-week appointment and returned at postnatal discharge; captures pulse, activity and prompts the daily check-in; syncs to the platform	Per device (lease or purchase) plus per-device service	A wearable that measures physiological parameters for clinical use is very likely a medical device ; Vytalix should procure a UKCA/CE-marked device from an existing manufacturer and integrate it, rather than manufacture (see \$6)
ML-INTEGRATE	Integration pack	Interfaces to the provider's maternity record (e.g., BadgerNet, K2, Euroking), DHIS2 alignment for programmes, HL7/FHIR endpoints, single sign-on	Per interface, one-off plus annual	Standard software integration
ML-ANALYTICS	Programme analytics	Population dashboards, defaulter tracing, facility performance, exportable indicators	Per programme	Non-clinical analytics
ML-TRAIN	Training and e-learning	Role-based training for midwives, community health workers, operators; refresher modules; certification of completion (not accredited)	Per cohort or per learner	E-Learning pillar

Replacement versus integration: where a hospital has no digital maternity record, MaternaLink Core can act as the working record for the programme (replace); where one exists, MaternaLink integrates and never becomes the legal record (integrate). Price the integration pack accordingly.

2. Cost-to-serve (why the prices are what they are)

Cost line	UK per woman per year	Africa programme per woman per year	Notes
Hosting, storage, backups	£0.60–£1.20	£0.30–£0.60	UK region; Nigeria residency adds 20–40%
Messaging (SMS/WhatsApp)	£1.50–£3.00	£0.80–£2.00	~40 messages per pregnancy; Nigeria SMS via aggregator
Support and clinical-safety overhead (allocated)	£3.00–£5.00	£0.60–£1.20	Includes hazard-log maintenance, incident handling
Onboarding and training (amortised)	£2.00–£4.00	£0.50–£1.00	Higher touch in UK trusts
Payment/collection, compliance	£0.40	£0.20	
Total cost-to-serve	£7.50–£13.60	£2.40–£5.00	Floor prices below
Floor price (no contract below)	£14	£4	Pilot minimums in \$3 protect the floor

Watch hardware (if used): device £60–£140 unit cost for a suitable wearable from an established manufacturer (ESTIMATE), 24–30 month life, 10–15% loss/damage, refurbishment £8–£15 per cycle. Cost per pregnancy served

≈ £35–£70. Priced at lease £4–£7 per woman per month or purchase with service plan.

3. Price book A — United Kingdom (ESTIMATE)

Per product

Product	Pricing model	Indicative price	Notes
ML-CORE	Site licence + per enrolled woman per year	Site licence £12,000–£30,000/year (by births: <3,000 / 3,000–6,000 / >6,000) plus £18–£36 per woman per year	A 5,000-birth trust at £22/woman ≈ £110k + £20k licence = £130k/year
ML-TRIAGE	Per site per year	£15,000–£35,000	Includes rule editor, two configuration workshops
ML-ENGAGE	Per enrolled woman per year	£8–£14	Bundled discount 20% with Core
ML-GLUCOSE	Per enrolled woman with condition per year	£20–£30	Typically 5–10% of the cohort
ML-VOICE	Per enrolled woman per year	£3–£5	Storage included up to 60 minutes per pregnancy
ML-WATCH	Lease per device per month; or purchase	£4–£7 per month per device (24-month term) or £120–£220 purchase + £30/year service	Includes replacement pool 10%
ML-INTEGRATE	One-off per interface + annual	£15,000–£40,000 setup; £4,000–£8,000/year	Maternity record vendors may charge their own fees (TO VALIDATE)
ML-ANALYTICS	Per site per year	£6,000–£12,000	Included in Enterprise bundle
ML-TRAIN	Per cohort (up to 25)	£2,500–£4,500	E-learning licence £40–£90 per learner per year
Implementation	One-off	£25,000–£75,000 per site	Clinical-safety case, DPIA support, configuration, go-live
Pilot (12 weeks, one site, up to 300 women)	Fixed	£40,000–£90,000	Includes evaluation report; converts to annual on success

Annual service plans (UK)

Plan	Price (% of annual licence)	Includes
Standard	18–22% (minimum £8,000)	Business-hours support, quarterly releases, security patching, hazard-log maintenance, uptime target 99.5% (design target, not a guarantee until measured)
Enhanced	25–28%	Standard plus extended hours 07:00–22:00, named account manager, two configuration changes per quarter, quarterly service review, DSPT evidence pack
Premium	30–35%	Enhanced plus 24/7 severity-1 response, clinical-safety officer on-call, annual clinical-safety case refresh, dedicated environment option, uptime target 99.9%

Enterprise bundles

Bundle	Contents	Indicative annual (5,000-birth trust)
Coordinate	Core + Engage + Standard service	£150k–£190k
Coordinate + Triage	Above + Triage + Analytics + Enhanced service	£210k–£270k
Full programme	Above + Glucose + Voice + Watch lease for 20% of cohort + Premium service	£330k–£420k

4. Price book B — Africa and programme funders (ESTIMATE)

Product	Pricing model	Indicative price	Notes
ML-CORE	Per enrolled woman per year, tiered by volume	£10 (up to 5,000) · £7 (5,001–25,000) · £5 (25,001–100,000) · £4 (100,000+)	Includes programme dashboard; site licence waived for public programmes
ML-ENGAGE (SMS/WhatsApp)	Per enrolled woman per year	£3–£5	Message costs passed through at cost above 40 messages
ML-TRIAGE	Per facility per year	£1,500–£4,000	Facility-level rules owned by the programme's clinical lead
ML-GLUCOSE	Per enrolled woman with condition	£6–£10	
ML-VOICE	Per enrolled woman	£1–£2	Local-language transcription roadmap
ML-WATCH	Per device	Lease £2–£4/month or purchase £90–£160 + £15/year service	Only where a programme funds hardware; not in base offers
ML-INTEGRATE (DHIS2 / HMIS)	One-off + annual	£8,000–£25,000 + £3,000/year	
ML-TRAIN	Per cohort of 30	£1,200–£2,500	Train-the-trainer model
Implementation	One-off per state or programme	£40,000–£120,000	Includes data-protection registration support, local hosting decision, baseline evaluation design
Annual service	20% Standard · 28% Enhanced · 35% Premium of software value	Minimum £12,000	In-country support partner required above 25,000 women (partner margin included)

Worked example: a 15,000-woman state programme (year 1)

Line	Amount
Core 15,000 × £7	£105,000
Engage 15,000 × £4	£60,000
Triage 20 facilities × £2,500	£50,000
Implementation (one-off)	£80,000
Training 10 cohorts	£18,000
Enhanced service (28% of £215k software)	£60,200

Line	Amount
Year-1 total	£373,200 (≈ £25 per woman)
Year-2 run-rate	≈ £275,000 (≈ £18 per woman)

Compared with £4.5m for the same cohort in the retired proposal. Gross margin at these prices ≈ 62% year 1, 74% year 2 (ESTIMATE).

Worked example: a 5,000-birth UK trust, Coordinate + Triage bundle

Line	Amount
Core licence £20,000 + 5,000 × £22	£130,000
Engage 5,000 × £10	£50,000
Triage	£25,000
Analytics	£8,000
Enhanced service (26% of £213k)	£55,400
Implementation (one-off, year 1)	£45,000
Year-1 total	£313,400 · year-2 run-rate £268,400

5. Commercial rules

- Pilot first, always: a fixed-price pilot with an evaluation report, converting to an annual contract with pilot fees credited 50% against year one.
- Minimum contract value: £40,000 (UK), £25,000 (programme), to protect the floor and service quality.
- Multi-year: 5% discount for 3-year terms; annual uplift capped at CPI + 2%.
- Volume discounts only via the published tiers; no ad hoc discounting below floor.
- Payment: UK annual in advance (public sector on receipt of PO); programmes 40% on signature, 40% on go-live, 20% at 6 months, or donor schedule.
- Hardware: never bundle device cost into the per-woman software price; show it as its own line so software margin is visible to investors and price comparisons to competitors are fair.

6. The Maternity Watch: build, buy or partner

Option	Pros	Cons	Verdict
Manufacture a Vytalix device	Brand, margin	Medical-device manufacturer obligations (UKCA, QMS ISO 13485, post-market surveillance), £1m+ and 24+ months	KILL for now
Integrate an existing UKCA/CE-marked wearable (e.g., a clinically certified wristband) under a supply agreement	Fast, compliant, priced as a pass-through	Lower margin; dependency	BUILD (partner) once v1 software is live
Consumer wearable (non-medical) for prompts and activity only	Cheap	Cannot be used for clinical vitals; expectation mismatch	TEST for engagement-only use (reminders, daily check-in)

7. Validation plan (before any price is quoted externally)

1. Three UK maternity digital leads: reaction to £18–£36 per woman and to bundles (Day 31–60 discovery interviews).
2. Two Nigerian programme leads and one NGO: reaction to £4–£10 per woman and the £25-per-woman year-1 all-in.
3. One insurer/HMO: appetite for per-member pricing.
4. Cost-to-serve check after the prototype audit (hosting, messaging actuals).
5. Regulatory opinion on Triage, Glucose and Watch modules before they are priced in any proposal.

Priorities / Risks / Next actions

Priorities: Core + Engage pricing validated first; Triage and Glucose only after the regulatory opinion; Watch as a partner device.

Risks: buyers anchor on free donor-funded programmes (mitigation: pilot-to-contract with evidence; complementarity positioning); under-pricing service (mitigation: minimums); device classification creep.

Next actions: put this price book into `/assets/PRICING_STRUCTURE.md` as the MaternaLink section; add price fields to the proposal template; test the two worked examples in the Day 31–60 interviews.



STRATEGY & OPERATIONS

VYTALIX FIRST 30 DAYS (Mon 14 September – Tue 13 October 2026)

Vytalix company-build documentation · reference 19_FIRST_30_DAYS

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

DATE

8 September 2026

VERSION

1.0 · Confidential draft

Contents

Executive view	1
0. How to read this plan	2
1. Calendar at a glance	3
2. Investor-call readiness tracker (preparation only until Day 31)	4
3. Day-by-day plan	5
4. What "done" looks like on Day 30 (PROPOSED scorecard)	6
5. Thirty-day cash budget (ESTIMATE; subset of /docs/18_90_DAY_EXECUTION_PLAN.md §8)	7
Priorities / Risks / Next actions	8

Owner: Founder (CEO/COO), with the Chief of Staff view · **Status:** PROPOSED · **Version:** 1.0 (8 September 2026) · **Day 1 = Monday 14 September 2026** · **Governed by:** `/FACTS_BASE.md` (language rules apply to every deliverable named here) · **Parent plan:** `/docs/18_90_DAY_EXECUTION_PLAN.md` (this document is the day-by-day detail of its Phase 1, Days 1–7, and Phase 2, Days 8–30; task numbers 1.x and 2.x below refer to that plan) · **Related:** `/docs/00_MASTER_AUDIT.md` §9 (immediate actions), `/docs/02_GROUP_STRUCTURE.md` §3 and §7 (IP options; name clearance), `/docs/17_FOUNDER_OPERATING_SYSTEM.md` (daily shape and rituals), `/docs/20_DECISION_FRAMEWORK.md`, `/investors/CALL_LISTS.md`, `/investors/OUTREACH_ENGINE.md`, `/assets/CONTENT_CALENDAR.csv`

Executive view

1. Thirty working-and-weekend days that take Vytalix from "a name, a brief and an undocumented prototype" (audit score 11/100, VERIFIED) to a formed, insured, contract-ready company with a signed IP position, a first paid engagement and investor materials that can be used honestly. Nothing in this plan is a record of achievement; every date is a target and every figure is an ESTIMATE unless labelled otherwise.
2. Day 1 carries the four blockers at once: name and trademark clearance (searches run the same day), the incorporation decision (gate G-A, decided Day 1, executed by Day 12), the David Agunede conversation and IP heads of terms (meeting Day 3, standstill Day 5, heads of terms Day 7, deed by Day 30), and preparation of the first ten investor calls (pack only — no outreach until the five pre-conditions in `/investors/OUTREACH_ENGINE.md` are logged as met, which is Day 31 at the earliest).
3. Revenue work starts Day 4 (first outreach touches) and runs every weekday from Day 8 as the first block of the day; full quota (40 contacts, 20 conversations, 5 discovery calls, 2 proposals per week) from Day 15; first signed engagement and first invoice due Day 30. Services are the cash story; MaternaLink (in development) is the equity story and is re-scoped to non-diagnostic v1 on Day 3 (gate G-C).
4. Weekends are light ($\leq 2-3$ hours): review, reading solicitor drafts, planning the following week. No outreach, no publishing at weekends. The one exception is Day 7 (Sun 20 Sept), when the IP heads of terms go to David Agunede because the parent plan fixes that date.
5. Cash cost of the 30 days: £6.5k–£15k (ESTIMATE; solicitor, attorney opinion, incorporation, ICO, insurance, accountant, tooling, prototype audit, domain). Funding is founder capital (TO CONFIRM) plus the first engagement deposit. If founder capital is below £12k, the prototype audit and insurance are held to P0 scope and everything else is deferred per `/docs/18_90_DAY_EXECUTION_PLAN.md` §8.

0. How to read this plan

- **Labels.** KNOWN / VERIFIED = fact; ASSUMPTION / ESTIMATE / PROPOSED / TO VALIDATE = not yet fact. Every number in a deliverable inherits the label of its source.
- **Daily shape (weekdays).** From `/docs/17_FOUNDER_OPERATING_SYSTEM.md` §2.1: 08:00 start-up and three priorities (one is always revenue) → 08:30–11:00 revenue block → 11:15–12:45 priority block (this plan's legal, entity and product items go here) → 12:45–13:30 LinkedIn → 13:30–15:30 meetings → 15:30–17:00 deep work → 17:00 shutdown and log. Kemek Enterprise work only after 17:30; Vytalix share $\geq 70\%$ of the week.
- **Gates.** G-A Incorporate · G-B IP resolution · G-C MaternaLink re-scope · G-D Ekiti go/no-go (Day 60, outside this plan; its inputs start here). Definitions in the parent plan §2.
- **Investor pre-conditions (all five must be logged before any investor call, email or LinkedIn message):** (1) incorporated; (2) MaternaLink IP heads of terms signed (deed executed or licence documented); (3) 2025

clinical-AI claims retired from all materials; (4) one-pager and data room v1 ready; (5) SEIS/EIS advance assurance submitted ("applied for", never "approved"). Pre-condition check is Day 31 (Wed 14 Oct).

- **The 2025 deck rule.** From Day 3 nothing external may say "predicts pre-eclampsia, sepsis...", "£36m", "expand to multiple NHS trusts", "NHS-approved", "clinically validated" or "government partner". "MaternaLink (in development)" is the only permitted product description.
- **Content.** Posts referenced by number are the titles in `/docs/14_MARKETING_AND_CONTENT_ENGINE.md` §6–§7 and the rows in `/assets/CONTENT_CALENDAR.csv`; Tuesday = insight, Wednesday = build-in-public, Thursday = practical.

1. Calendar at a glance

Day	Date	Theme	Gate / hard deadline
1	Mon 14 Sept	Four blockers opened; operating rhythm installed	G-A decided; name searches run
2	Tue 15 Sept	CRM live; LinkedIn profile; solicitor and accountant shortlists; Post 1	—
3	Wed 16 Sept	David Agunede conversation; intended-use statement adopted; Post 2	G-C adopted
4	Thu 17 Sept	Attorney opinion; solicitor selected; first 20 outreach touches; Post 3	—
5	Fri 18 Sept	Name decision; standstill signed; solicitor and accountant instructed; domain; 40 touches; investor pack v1; Friday review 1	Clearance memo; standstill
6	Sat 19 Sept	Light: review, read heads-of-terms draft	—
7	Sun 20 Sept	Light: IP heads of terms sent; Week 2 plan	G-B step 1 (heads of terms sent)
8	Mon 21 Sept	Incorporation filing prepared; company page created; prototype audit brief	—
9	Tue 22 Sept	Contract templates to solicitor; insurance quotes; discovery calls 1–2; Post 4	—
10	Wed 23 Sept	Incorporation filed (target) ; founder IP assignment signed; Post 5	G-A execution target
11	Thu 24 Sept	Bank application; company page live; proposal 1; Post 6	—
12	Fri 25 Sept	Incorporation hard deadline ; templates sent; prototype audit started; Friday review 2	G-A hard deadline (Day 12)
13	Sat 26 Sept	Light: Article A1 outline	—
14	Sun 27 Sept	Light: Week 3 plan; 5 calls / 2 proposals check	Task 2.6 target
15	Mon 28 Sept	Full sales quota begins; ICO registration; website legal pages	—

Day	Date	Theme	Gate / hard deadline
16	Tue 29 Sept	Insurance decision; advisory long-list; SEIS/EIS drafting begins; Post 7	—
17	Wed 30 Sept	Heads-of-terms check-in; Search Console; website QA; Post 8	—
18	Thu 1 Oct	Grant calendar; data room index; Post 9	—
19	Fri 2 Oct	Bank, ICO, insurance, newsletter tool, interview list all due ; heads-of-terms response date; Friday review 3	Tasks 2.2, 2.3, 2.4, 2.19 (list)
20	Sat 3 Oct	Light: Article A1 edit	—
21	Sun 4 Oct	Light: Week 4 plan; pricing memo draft	—
22	Mon 5 Oct	Pricing re-base memo adopted ; Ekiti call requested; advisory approaches 1–3	Task 2.9
23	Tue 6 Oct	Article A1 published (LinkedIn); deck v1 drafting; Post 10	—
24	Wed 7 Oct	Partial Month 0 close; UK interviews 1–2; prototype audit draft; Post 11	—
25	Thu 8 Oct	Newsletter issue 1; event 1 (if diary allows); first-engagement negotiation; Post 12	—
26	Fri 9 Oct	Website live; Ekiti status call; templates returned; data room v1; SEIS/EIS drafted; grant calendar ; Friday review 4	Tasks 2.7, 2.12, 2.14–2.17, 2.20
27	Sat 10 Oct	Light: read prototype audit; board pack	—
28	Sun 11 Oct	Light: Week 5 plan; Day 30 checklist; board agenda	—
29	Mon 12 Oct	IP deed signing scheduled; SOW push; investor pack re-verification	—
30	Tue 13 Oct	First signed engagement; IP deed executed; prototype audit report; SEIS/EIS submitted; first board meeting; pre-condition status logged	Phase 2 exit; tasks 2.8, 2.10, 2.11, 2.17, 2.18

2. Investor-call readiness tracker (preparation only until Day 31)

Pre-condition	Owner	Target date in this plan	Evidence to log
1. Incorporated	Founder + accountant	Wed 23 Sept (hard Fri 25 Sept)	Certificate of incorporation, company number
2. IP heads of terms signed; deed executed or licence documented	Founder + solicitor	Heads of terms sent Sun 20 Sept; response Fri 2 Oct; deed Tue 13 Oct	Signed heads of terms; signed deed or licence
3. 2025 claims retired from all materials	Founder	Wed 16 Sept	Claims-check log; retired-deck note

Pre-condition	Owner	Target date in this plan	Evidence to log
4. One-pager and data room v1	Founder	Fri 9 Oct	Data room index; one-pager v1
5. SEIS/EIS advance assurance submitted	Founder + accountant	Tue 13 Oct	HMRC submission receipt

The first ten calls (PROPOSED order; from `/investors/CALL_LISTS.md` Call List 1, phone-first only where a public number was found and sourced during research; re-verify every route on Day 29 before use):

#	Investor (Call List 1 rank)	Route	Note
1	SBRI Healthcare (rank 1)	Verified public office number	Funder, not equity; competition themes TO VALIDATE against open calls
2	Innovate UK (rank 3)	Verified public helpline	Non-dilutive; eligibility requires the incorporated entity
3	NIHR i4i (rank 5)	Verified public number	Research-track relevance; clinical partner likely required (TO VALIDATE)
4	TLcom Capital (rank 6)	Verified London and Lagos numbers	Africa health mandate; UK-registered
5	AlbionVC (rank 8)	Verified office number	UK healthtech practice
6	Grand Challenges Canada (rank 9)	Verified number; open calls via portal	Maternal and newborn health mandate
7	Ventures Platform (rank 10)	Verified Lagos number	Nigeria; health tech named sector
8	GC Angels (rank 12)	Verified number	Confirm geographic eligibility first (North West focus, TO VALIDATE)
9	o2h Ventures (rank 14)	Published contact number on official page	SEIS/EIS health investor
10	Cambridge Enterprise Seed Funds (rank 13)	Verified number	Confirm eligibility (Cambridge affiliation may be required, TO VALIDATE) before calling

Octopus Ventures, Ada Ventures, Goddess Gaia Ventures and Nina Capital (ranks 2, 4, 7, 11) have no public phone number and are approached email/portal-first in Week 6 per `/docs/18_90_DAY_EXECUTION_PLAN.md` task 3.3. Scripts: `/investors/OUTREACH_TOP25.md`. Never call a personal mobile.

3. Day-by-day plan

Day 1 — Monday 14 September 2026 · Open the four blockers

Action

1. Install the operating rhythm (task 1.1, 1.13): calendar blocks for the daily shape and the Friday review; daily log and decision log opened; `/operations/KPI_DASHBOARD.csv` and `/operations/PIPELINE_TRACKERS.csv` set as the two live trackers; time-allocation rule ($\geq 70\%$ Vytalix) adopted and Kemek Enterprise Ltd entered in the conflicts register. Log decision D-001.
2. Name and trademark clearance, steps 1–4 of `/docs/02_GROUP_STRUCTURE.md` §7.2 (task 1.2): Companies House availability and "too like" check for "Vytalix Group Ltd" and the reserve name "Vytalix Health"

Technologies Ltd"; UK IPO word and phonetic search in Classes 9, 35, 41, 42, 44 for "Vytalix", "Vytalyx", "Vitalix"; EUIPO and USPTO knock-out search; domain and handle audit (vytalix.com / .co.uk / .health / .io; LinkedIn, X, Instagram). Run the same five-step check for "MaternaLink" and "Maternal Link" (Classes 9, 42, 44). Brief a trade-mark attorney for the 30-minute opinion (step 5) on Thu 17 Sept; budget £300–£600 (ESTIMATE).

3. Incorporation decision, gate G-A (task 1.3): decide to incorporate one English company now (not a group), sole director, registered office (accountant's address or founder's, TO DECIDE), share capital per accountant's advice; decision D-002 logged with the rule "incorporate under the cleared name; if clearance fails, incorporate under the reserve name within five days rather than delay". Shortlist three accountants (SEIS/EIS experience, Xero, fixed monthly fee) and request calls for Day 2.
4. David Agunede (task 1.4): send a meeting request for Wed 16 Sept with a written agenda (his role and intent; authorship of the code, decks and proposals; who hosts and pays for the Vercel prototype; whether any real patient data has ever been entered; the Ekiti status since 18 March 2026; any other conversations in the "Maternal Link" name). Attach the IP audit questionnaire (/docs/15_LEGAL_AND_REGULATORY.md §8.3). Draft the 60-day mutual standstill and confidentiality letter (/docs/02_GROUP_STRUCTURE.md §3.2) for solicitor review. Draft the IP heads of terms internally from §3.1 (Option 1 assignment-for-equity as the opening position; Option 2 exclusive licence as fallback) — internal only; no numbers are offered to anyone until a solicitor has reviewed them.
5. First-ten-investor-call preparation (task 1.8): re-read /investors/TOP_25_INVESTORS.md and Call List 1; pick the ten names in §2 above; open the call-pack document with the header "NOT FOR USE UNTIL PRE-CONDITIONS 1–5 ARE LOGGED AS MET"; list the pre-conditions with their owners and dates.
6. Internal note that the 2025 "Black Maternal health" deck and the £36m Ekiti headline are withdrawn from external use pending the formal adoption of the intended-use statement on Day 3.

Why — Every downstream item (bank, ICO, insurance, SEIS/EIS, contracts, grants, investor outreach, website) depends on entity and name; every investor and buyer conversation depends on being able to say what Vytalix owns; every public statement depends on retiring the over-claims. Opening all four on Day 1 means no gate waits on a founder decision.

Deliverable — Decision log entries D-001 and D-002; clearance search log with screenshots and dates (each result labelled VERIFIED with source); meeting invite and questionnaire sent to David Agunede; standstill letter draft; heads-of-terms draft (internal); accountant shortlist; investor call pack v0 with ten names and routes; three daily priorities written and closed in the log.

Expected result — By 17:30 the founder knows whether "Vytalix" is available at Companies House and whether an identical or confusingly similar live UK mark exists in Classes 9, 42 or 44; the David Agunede meeting is booked; the attorney opinion is scheduled; the rhythm exists for every later day.

Day 2 — Tuesday 15 September 2026 · CRM, profile, professional bench

Action

1. Create HubSpot Free CRM (task 1.9): import the deal stages and required fields from /docs/13_SALES_ENGINE.md §3; create the loss-reason codes. Build the first 50 of the 150-contact ICP list (segments S1 and S2 first), each with a source URL and a lead score.
2. Rewrite the founder LinkedIn profile (task 1.10): headline "Founder, Vytalix — health-technology advisory, consulting and MaternaLink (in development) · Digital maternity, UK and Nigeria" (PROPOSED wording);

About section with the stage disclosure ("Pre-seed, founder-led. MaternaLink is in development."). Publish Post 1 ("Why I'm starting a health-tech group with a consultancy, not an app") in the 12:45 slot.

3. Solicitors: shortlist three fixed-fee start-up practices; send the scope (founder IP assignment; David Agunede IP deed and shareholders' agreement terms; MSA/SOW/NDA/contractor templates; name-risk opinion; financial-promotion wording later) with a fee cap of £4k–£9k (ESTIMATE); ask for a Day 4 decision call.
4. Accountant calls (three): SEIS/EIS advance-assurance experience, Xero, Corporation Tax, registered-office service, share-capital advice; decide on Day 3.
5. Prepare the David Agunede meeting brief: the VERIFIED facts from `/FACTS_BASE.md` §1, the questions from Day 1, the three IP routes and why assignment is preferred, the standstill letter, and the sentence to use if he asks for numbers ("a solicitor is being instructed this week; I will put written terms in front of you by Sunday").
6. Start the 13-week cash-flow v0 (task 1.12): founder capital available (TO CONFIRM), the 30-day cost list in \$5 below, first-engagement deposit scenario.

Why — The sales engine cannot run without a list and a CRM; the public presence must be stage-honest before the first outreach touch lands; the legal and accounting bench must be in place before the Day 3 conversation produces anything that needs drafting.

Deliverable — CRM live with stages and fields; 50 contacts with source URLs; LinkedIn profile updated; Post 1 published; solicitor quotes requested; accountant call notes; meeting brief; cash-flow v0 skeleton.

Expected result — Profile views begin to rise (target 200 in Week 1, PROPOSED); the founder walks into Day 3 with facts, questions and a written standstill letter, not an improvised conversation.

Day 3 — Wednesday 16 September 2026 · The David Agunede conversation; gate G-C

Action

1. Hold the David Agunede conversation (task 1.4), 60 minutes, notes taken in real time and written up the same day. Facts to establish: his role and what he wants; who wrote the code and the decks; hosting, accounts and access for the Vercel prototype; whether any real patient data has ever been entered; what happened after the 18 March 2026 Ekiti email (any reply, meeting, budget line, or nothing); any other government, NGO or investor conversation held in the "Materna Link" name; his availability and appetite for a defined role. Present the standstill and confidentiality letter (60 days; neither party licenses, sells or presents MaternaLink to third parties while terms are agreed) and ask for signature by Fri 18 Sept. Explain the three routes and Vytalix's preference for assignment-for-equity with vesting; do not state a percentage. Agree that written heads of terms arrive on Sun 20 Sept and that his response is due Fri 2 Oct (PROPOSED). Agree that any Ekiti contact from now on is joint and factual (status only), under the standstill.
2. Adopt the MaternaLink v1 intended-use statement, gate G-C (task 1.7): the statement in `/docs/04_MATERNALINK_PRODUCT_STRATEGY.md` §21.1 is logged as decision D-003 (PROPOSED; TO VALIDATE with regulatory counsel). Retire the 2025 deck, the £36m headline and "expand to multiple NHS trusts" from all external use; run the claims check over every file in `/website`, `/assets` and `/investors`; label every product reference "MaternaLink (in development)". Move the AI risk score to the v3 research track in every document.
3. Publish Post 2 ("Day 1: what Vytalix is, what it isn't, and what I don't know yet").
4. Continue the ICP list to 100 contacts. Select the accountant; request the engagement letter for Day 5.

Why — Problem 1 in the master audit (IP undocumented) and problem 3 (over-claimed positioning) are the two items that would end an investor or trust conversation in minutes. Facts first, then terms; and the claims discipline has to be in place before the company says anything in public about the product.

Deliverable — Conversation note (facts labelled KNOWN; his statements labelled as his statements); standstill letter handed over; decision D-003; claims-check log with zero open items; Post 2; accountant selected; ICP list at 100.

Expected result — Vytalix knows what exists, who made it and what he wants; the standstill is on its way to signature; every external artefact is stage-honest. If the conversation reveals third-party code, real patient data or a live Ekiti commitment, the prototype audit brief (Day 8) and the solicitor instruction (Day 5) are amended the same evening.

Day 4 — Thursday 17 September 2026 · Attorney opinion; solicitor chosen; outreach begins

Action

1. Trade-mark attorney opinion call (clearance step 5) on the four collisions in `/docs/02_GROUP_STRUCTURE.md` §7.1 (Vytalix Inc., Vytalize Health, VITALIX LTD ×2, plus VITALX and VYTAL(UK) as low-risk); request the written memo (proceed / proceed with risk / change) by Fri 18 Sept.
2. Select the solicitor; send the instruction letter and the Day 3 conversation note; ask for the standstill letter to be finalised for signature and the heads of terms to be reviewed for Sunday.
3. Send the standstill letter to David Agunede for signature (solicitor-checked version).
4. Revenue block: send the first 20 outreach touches (templates E1/L1, one personalised reason each) to A-scored contacts; log every touch (task 1.10 starts).
5. Publish Post 3 ("The three questions I ask before any digital maternity idea"); 5 substantive comments on target-audience posts.
6. Investor pack: draft the stage-honest one-pager outline (what exists, what does not, what is being raised and for what) and copy the objection table from `/investors/OUTREACH_ENGINE.md` §8 into the pack. Complete the ICP list to 150 with source URLs.

Why — The name decision on Day 5 needs a professional opinion, not a founder's guess; the first touches must go out this week because the first signed engagement is due Day 30 and the sales cycle for S1 is 2–6 weeks (ESTIMATE).

Deliverable — Attorney memo requested (received Day 4 or 5); solicitor engagement letter; standstill sent for signature; 20 touches logged; Post 3; one-pager outline; 150-contact list.

Expected result — Three or more replies to the first touches by Day 10 (PROPOSED KPI); the founder has a written basis for the name decision tomorrow.

Day 5 — Friday 18 September 2026 · Week 1 close: name decision, bench instructed, standstill signed

Action

1. Name decision (task 1.2, decision D-004) under the rule in `/docs/02_GROUP_STRUCTURE.md` §7.2: if Companies House and UK IPO are clean and the attorney says "low risk", proceed with "Vytalix Group Ltd" and instruct the trade-mark filing for Week 7 (task 3.13); if any step fails, adopt the reserve name for

incorporation and keep "Vytalix" as a trading name only if the attorney supports it; otherwise rename. Repository spelling "Vitalyx" retired regardless.

2. Within 24 hours of the decision (task 1.11): register the domain(s) and social handles; set up Google Workspace on the domain; forward the Gmail address for continuity.
3. Solicitor and accountant instructed (tasks 1.3, 1.6): both engagement letters signed; solicitor briefed on the Option 1 / Option 2 heads of terms for Sunday.
4. Standstill and confidentiality letter signed by both parties (task 1.4). If unsigned, log the reason and escalate on Day 8; the Day 7 heads of terms still go out.
5. Revenue block: touches 21–40 sent (task 1.10 complete); replies answered within 24 hours; discovery calls booked for Days 9–12.
6. Finance baseline (task 1.12): founder capital figure logged (TO CONFIRM); ring-fenced founder account opened pending the company account; 13-week cash-flow v0 complete.
7. Investor pack v1 (task 1.8): ten names with verified routes and the per-investor scripts from `/investors/OUTREACH_TOP25.md`; one-pager outline; objections; pre-condition tracker — marked "not for use".
8. Content batch (Friday 14:00): draft posts 4–6 for Week 2, claims-checked. Pipeline clean and KPI update (15:00); Friday review 1 (15:30, template in the founder OS §5); cash check (16:15); time audit line (Vytalix hours vs Kemek).

Why — Phase 1 exit criteria in the parent plan: standstill signed, name decision made, claims retired, intended-use adopted, CRM live, 40 first touches sent; heads of terms sent by Day 7. All but the last close today.

Deliverable — Clearance memo on file; decision D-004; domain and Workspace live; two engagement letters; signed standstill; 40 touches logged; cash-flow v0; investor pack v1; three posts drafted; Friday review 1 written to `/operations/reports/`.

Expected result — The company can be filed the moment the accountant confirms the paperwork; the founder has a professional email address before the first proposal; David Agunede cannot present MaternaLink elsewhere during negotiation; the week's three commitments for Week 2 are written down.

Day 6 — Saturday 19 September 2026 · Light day

Action — Read the solicitor's reviewed heads of terms and mark questions (≤ 60 minutes); read the Week 1 review; note what slipped. No outreach, no publishing.

Why — Sunday's send must be a document the founder has read line by line; the week's slippage must be visible before Monday's plan.

Deliverable — Marked-up heads of terms; a short slippage list.

Expected result — Nothing goes to David Agunede that the founder cannot explain in plain words.

Day 7 — Sunday 20 September 2026 · Heads of terms sent; Week 2 planned

Action

1. Send the IP heads of terms to David Agunede (task 1.5; gate G-B step 1): Option 1 assignment-for-equity (8–12% ordinary shares, PROPOSED range from `/docs/02_GROUP_STRUCTURE.md` §3.1, reverse vesting four

years with a twelve-month cliff, warranties on authorship, no real patient data, brand and domains transferred, all government conversations in Vytalix's name, defined role if he wants one); Option 2 exclusive licence with an option to acquire as the fallback. Response date Fri 2 Oct (PROPOSED). Copy the solicitor. If the solicitor's review lands late, send first thing Monday — the parent plan's deadline is Day 7 and a Monday-morning send is a logged one-day slip, not a failure.

2. Write the Week 2 plan (task 1.14): targets for contacts, conversations, calls and proposals; incorporation filing steps; content for the week.

Why — Written terms on the table before Phase 2 begins is the only way the Day 30 deed date holds.

Deliverable — Heads of terms sent (email timestamp logged); Week 2 plan in the daily log.

Expected result — G-B is in motion with a response date; Week 2 starts with a plan, not a backlog.

Day 8 — Monday 21 September 2026 · Phase 2 opens: entity paperwork, company page, prototype audit brief

Action

1. Monday weekly plan (08:00): CEO dashboard read; week targets set; calendar blocked.
2. Incorporation filing prepared with the accountant (task 2.1): SIC codes, registered office, person with significant control, articles (solicitor confirms they are vesting-ready and allow the later option pool), share capital, director's service address; founder IP assignment deed drafted by the solicitor for signature on the filing day.
3. LinkedIn company page created for the decided name (task 2.13) with the stage-honest About text; not yet announced.
4. Prototype audit brief written (task 2.10; `/docs/04_MATERNALINK_PRODUCT_STRATEGY.md` \$25 and SOP 08): authorship and chain of title, third-party licences, secrets in code, data held (any real data is a stop-and-report event), hosting accounts, security posture, reuse-versus-rebuild recommendation. Shortlist three fractional CTOs; budget £1,000–£2,500 (ESTIMATE). Request repository and hosting access from David Agunede under the standstill.
5. Revenue block from today: 8 new contacts, 5 follow-ups, one discovery call or meeting attempt every weekday (daily non-negotiables); discovery calls 1–2 confirmed for Day 9.
6. First quarterly partnership-planning session (SOP 07 trigger, week of 21 Sept; 30 minutes): read the partner long-list; mark the three organisations to approach after incorporation. No approaches yet.

Why — Filing on Day 10 needs every field ready on Day 8; the audit needs access and a brief before an auditor is engaged; the revenue rhythm must be daily from here because the Day 14 target is five discovery calls and two proposals.

Deliverable — Filing pack complete; founder IP assignment in draft; company page created; audit brief; CTO shortlist; access request sent; daily log with revenue block completed.

Expected result — Nothing stands between the accountant and a Day 10 filing except the name decision already made on Day 5.

Day 9 — Tuesday 22 September 2026 · Contracts, insurance, discovery calls 1–2

Action

1. Discovery calls 1 and 2 (30-minute script, [/docs/13_SALES_ENGINE.md](#) §5); Problem / Budget / Authority / Timeline / Success criteria captured in the CRM within 30 minutes.
2. Contract templates compiled from [/docs/15_LEGAL_AND_REGULATORY.md](#) §9.1 (MSA, SOW, NDA, contractor agreement with IP clause) and queued for the solicitor by Day 12 (task 2.7). Any engagement signed before incorporation goes on a personal contract with a novation clause — solicitor to confirm.
3. Professional indemnity and public liability quotes requested from a broker (task 2.4); insurance must be bound before the first paid engagement.
4. Advisory-board long-list started (task 2.15): 12 names across the six profiles in [/docs/02_GROUP_STRUCTURE.md](#) §4.2, obstetrics/midwifery and regulatory/clinical-safety first; public roles only; no approach before incorporation.
5. Publish Post 4 ("Ockenden and Kirkup keep finding the same failure: communication", both reports cited).

Why — The first proposal (Day 11) needs a contract path; the first engagement needs insurance; credibility needs advisers, and advisers need a company to advise.

Deliverable — Two discovery-call records; template pack ready for the solicitor; broker quote request; long-list of 12; Post 4.

Expected result — At least one discovery call converts to a proposal decision within 48 hours (SOP 01 step 8).

Day 10 — Wednesday 23 September 2026 · Incorporation filed (target); founder IP assigned**Action**

1. File the incorporation online (task 2.1; fee ~£100, ESTIMATE, TO VALIDATE current Companies House fee); founder IP assignment deed signed the same day (all Vytalix materials created to date, including this repository, assigned to the company on incorporation); statutory registers opened; decision D-005 logs the filing.
2. David Agunede follow-up: confirm receipt of the heads of terms; answer questions in writing, copying the solicitor; no revised numbers without solicitor advice.
3. Fractional CTO selected for the prototype audit; engagement letter (fixed fee); start date Day 12 subject to access.
4. Discovery call 3. Publish Post 5 ("We are calling the AI risk score a research track. Here's why.").
5. ICP list refreshed with replies and dead contacts coded; follow-ups at day 4 for Week 1 touches.

Why — Gate G-A execution. Same-day incorporation is usual for digital filings (ASSUMPTION; can take longer), so filing on Day 10 leaves two working days before the Day 12 hard deadline.

Deliverable — Filing submitted (and certificate, if issued the same day); signed founder IP assignment; CTO engagement letter; discovery-call record 3; Post 5.

Expected result — Vytalix exists as a legal person by Day 12; the audit can start the moment access is granted.

Day 11 — Thursday 24 September 2026 · Bank, company page live, proposal 1**Action**

1. On receipt of the certificate: open the business bank account application (digital bank; TO VALIDATE provider), start the Xero subscription, diarise Corporation Tax registration (within three months of trading), and send the company number to the solicitor, accountant and broker.
2. LinkedIn company page goes live (task 2.13) with one post: what the company is, what it is not, and the stage disclosure. Founder reposts.
3. Proposal 1 drafted from the package menu (A1 advisory days or C7 Grant & Investor Readiness Sprint are the expected first offers, `/assets/PRICING_STRUCTURE.md`), FACTS_BASE language check run, 14-day validity; sent today if the discovery call warrants it.
4. UK discovery-interview list started (task 2.19): 30 names (midwives, digital maternity leads, private providers, charities) via public routes; participant information sheet and consent form drafted for no-patient-data professional interviews.
5. Publish Post 6 ("A one-page intended-use statement — the most valuable document a health-tech founder can write"); 20 comments across the week reached by today.

Why — Revenue needs an invoice route (bank) and a contract path; the company page is the credibility surface for every proposal recipient who looks Vytalix up; UK market validation starts now so that 15 interviews are done by Day 60.

Deliverable — Bank application submitted; Xero live (trial); company page live; proposal 1 sent or ready; interview list of 30 with consent form; Post 6.

Expected result — The first proposal is in a buyer's inbox with a professional company presence behind it.

Day 12 — Friday 25 September 2026 · Incorporation hard deadline; audit starts; Friday review 2

Action

1. Incorporation confirmed (hard deadline, gate G-A). If the filing is rejected or delayed, the reserve-name route is executed today — not Monday.
2. Contract templates sent to the solicitor (task 2.7; return date Fri 9 Oct).
3. Prototype audit begins (task 2.10) if access has been granted; if not, log the refusal or delay as a G-B risk and raise it with the solicitor — an unaudited prototype cannot be valued in the deed.
4. Discovery calls 4 and 5 held (or firmly booked for Mon 28 Sept); proposal 2 drafted; sent today or on Day 15 at the latest (task 2.6 target: 5 calls, 2 proposals by Sun 27 Sept).
5. Content batch: posts 7–9; Article A1 ("DTAC without the panic") outline. Pipeline clean; KPI update; Friday review 2; cash check; time audit.

Why — Phase 2 has a hard date for the entity for a reason: every later deadline (bank Day 19, ICO Day 19, insurance Day 19, SEIS/EIS Day 30) counts from it.

Deliverable — Certificate of incorporation on file; templates with the solicitor; audit kick-off note; discovery-call records 4–5; proposal 2 draft; Friday review 2.

Expected result — Phase 2 milestones for Week 2 are all either done or have a dated remedial action in Monday's plan.

Day 13 — Saturday 26 September 2026 · Light day

Action — Article A1 outline expanded to section headings and sources (≤ 90 minutes); read proposals 1–2 once more as the buyer would. No outreach.

Why — A1 is the Week 4 long-form anchor and needs a start; proposals benefit from a cold read.

Deliverable — A1 outline with sources; proposal edits noted.

Expected result — Monday starts with writing, not planning.

Day 14 — Sunday 27 September 2026 · Light day; task 2.6 check

Action — Week 3 plan; check task 2.6 (five discovery calls, two proposals): if short, write the remedial plan (extra outreach, event, warm-intro asks); check the David Agunede heads-of-terms status against the Fri 2 Oct response date.

Why — The first signed engagement on Day 30 depends on proposals being out by Day 14.

Deliverable — Week 3 plan; 2.6 status line (met / short by n); G-B status line.

Expected result — The founder enters Week 3 knowing the gap, if any, and the action to close it.

Day 15 — Monday 28 September 2026 · Full sales quota; ICO; website legal pages

Action

1. Sales engine at full weekly quota from today (task 2.5): 40 new contacts, 20 conversations, 5 discovery calls, 2 proposals this week and every week after.
2. ICO registration (task 2.3; Tier 1 fee, TO VALIDATE current amount) submitted now that the company exists; legitimate-interests assessment for B2B outreach written; privacy notice drafted for the website and CRM.
3. Bank account application chased; if the first provider is slow, a second application is opened in parallel.
4. Website: entity details (company name, number, registered office) inserted into the `/website` footer, privacy, cookie and terms pages; staging review against `/docs/14_MARKETING_AND_CONTENT_ENGINE.md` §3.2; the 90-day plan's publication date is Fri 9 Oct (task 2.12); if legal pages, ICO reference and forms are ready earlier, publish earlier — never later than Day 26.
5. Newsletter tool shortlisted (free tier; double opt-in required; TO VALIDATE provider).
6. Event 1 selected for Week 4 (candidate: an autumn London NHS-digital or health-tech founder event; date and cost TO VALIDATE against organiser sites before booking).

Why — Lawful outreach and a lawful newsletter need the ICO registration; the website converts the LinkedIn audience only if it exists with real entity details.

Deliverable — ICO submission receipt; LIA and privacy notice drafts; website staging with entity details; newsletter tool chosen; event 1 booked or shortlisted; quota tracker started.

Expected result — Week 3 quota met; the site can go live as soon as forms and the ICO reference are in.

Day 16 — Tuesday 29 September 2026 · Insurance; advisory long-list; SEIS/EIS drafting; Post 7

Action

1. Insurance decision (task 2.4): compare quotes; bind PI and PL (annual, paid monthly if cash requires; £800–£2,000 ESTIMATE); policy schedule filed.
2. Advisory-board long-list complete (12 names, six profiles); approach letters drafted (public roles only; written consent before any name is used anywhere).
3. SEIS/EIS advance-assurance drafting begins with the accountant (task 2.17): trade description covering product R&D and services together; the HMRC expectation of an identified prospective investor is TO VALIDATE — if required, the application is submitted the day one exists.
4. Prototype audit mid-point check-in with the fractional CTO: any stop-and-report findings (real data, third-party code, exposed secrets)?
5. Discovery calls; proposals follow-up (+3 days). Publish Post 7 ("What DTAC actually asks for (and what it doesn't)").

Why — Insurance before delivery; advisers before pilots; SEIS/EIS "applied for" is pre-condition 5 for investor calls.

Deliverable — Policy schedule; long-list and draft approaches; SEIS/EIS draft started; audit check-in note; Post 7.

Expected result — No first engagement is delivered uninsured; the SEIS/EIS application is on track for Day 30.

Day 17 — Wednesday 30 September 2026 · Heads-of-terms check-in; Search Console; website QA; Post 8

Action

1. Heads-of-terms check-in call with David Agunede (15–30 minutes): questions, sticking points, whether Option 1 or Option 2 is the likely landing; solicitor briefed afterwards; response date Fri 2 Oct confirmed.
2. Google Search Console verified on the domain (possible pre-launch); sitemap and robots checked; schema.org Organization added with entity details.
3. Website QA: contact form routing and data-storage statement, cookie banner, accessibility pass, page speed under 2 seconds (static site rule), every page's stage disclosure and "MaternaLink (in development)" wording.
4. Discovery calls; quota tracking at mid-week (Wednesday 12:00 check). Publish Post 8 ("Naming a company when four similar names already exist" — tell the real Week 1 story, whatever the outcome was).

Why — G-B needs active management, not a waiting period; the site launch on Day 26 must not be delayed by QA findings.

Deliverable — Check-in note; Search Console verified; QA log with fixes; Post 8.

Expected result — The founder knows by tonight whether the deed can be drafted for Day 30 or whether the licence fallback needs drafting in parallel.

Day 18 — Thursday 1 October 2026 · Grant calendar; data room index; Post 9

Action

1. Grant calendar (task 2.20) started: Innovate UK Smart, SBRI Healthcare, NIHR i4i, Grand Challenges Canada — open calls, eligibility (UK-registered entity, clinical partner, match funding), deadlines; every entry TO

VALIDATE on the funder's site; complete by Day 26.

2. Data room v1 index started (task 2.16): one-pager, deck v1, financial-model summary from `/finance/Vytalix_Financial_Model_3yr.xlsx`, IP agreement summary (placeholder until signed), advisory board (empty until consented), pilot plan, risk register.
3. Proposal follow-ups (+7 days); new proposals from this week's calls.
4. Publish Post 9 ("Discovery interview guide: 12 questions for maternity teams", template); 20 comments for the week.

Why — Pre-condition 4 (data room and one-pager) is due Day 26; grants are the most accessible non-dilutive money for a newly incorporated company and their deadlines do not move for Vytalix.

Deliverable — Grant calendar draft; data room index v0; Post 9; proposal follow-up log.

Expected result — By Day 26 there is at least one eligible open call on the calendar (PROPOSED KPI) and a data room a solicitor can review.

Day 19 — Friday 2 October 2026 · Deadline cluster; heads-of-terms response; Friday review 3

Action

1. Due today (parent plan): business bank account open and Xero live (task 2.2); ICO reference number received or chased (2.3); insurance bound (2.4); newsletter tool live with double opt-in (2.12); UK discovery-interview list of 30 and consent form complete (2.19). Each is logged as done or as a dated remedial action.
2. David Agunede heads-of-terms response date (PROPOSED): outcome logged as one of — agreed in principle (solicitor drafts the assignment deed and shareholders'-agreement terms for Day 30); negotiating (points listed; second response date no later than Fri 9 Oct); declined assignment (Option 2 licence drafted); no response (chase; Day 60 hard stop and clean-room fallback remain in force).
3. Content batch: posts 10–12; Article A1 final draft; newsletter issue 1 draft (sections per `/docs/14_MARKETING_AND_CONTENT_ENGINE.md` §3.6).
4. Investor pack review against the five pre-conditions: status of each logged (this is a preparation step, not outreach).
5. Pipeline clean; KPI update; Friday review 3; cash check; time audit.

Why — Five infrastructure items converge here because each depends on the certificate from Day 10–12; the G-B response date decides which legal document is drafted for Day 30.

Deliverable — Bank and Xero confirmation; ICO receipt; policy schedule; newsletter tool live; interview list and consent form; G-B outcome note; content batch; Friday review 3.

Expected result — The company can invoice, insure and lawfully email; the IP route for Day 30 is known.

Day 20 — Saturday 3 October 2026 · Light day

Action — Edit Article A1 (≤ 90 minutes); read the week's review. No outreach.

Deliverable — A1 ready for a Monday final read.

Expected result — A1 publishes on Day 23 without a weekend rush.

Day 21 — Sunday 4 October 2026 · Light day; pricing memo draft

Action — Week 4 plan; draft the pricing re-base memo for adoption tomorrow (task 2.9): services price list confirmed from `/assets/PRICING_STRUCTURE.md`; MaternaLink (in development) indicative platform-layer range £6–£40 per woman per year (ESTIMATE) replaces the £300 figure everywhere; cost-to-serve model v0 (SMS pass-through, hosting, support, CHEW onboarding — all ESTIMATE).

Deliverable — Week 4 plan; pricing memo draft.

Expected result — Monday's adoption is a decision, not a drafting session.

Day 22 — Monday 5 October 2026 · Pricing memo adopted; Ekiti call requested; advisory approaches 1–3

Action

1. Pricing re-base memo adopted (task 2.9; decision D-006): find-and-replace across the repository, website and every proposal template; the £300 figure survives only in `/FACTS_BASE.md` as a VERIFIED historical proposal figure, labelled UNVALIDATED.
2. Ekiti status call requested for Fri 9 Oct (task 2.14): a factual call to learn the outcome of the March 2026 proposal; with David Agunede if agreed under the standstill; no new offer, no price, no restatement of the £4.5m proposal.
3. Advisory-board approaches 1–3 sent (task 2.15; obstetrics/midwifery and regulatory/clinical-safety profiles first); each asks for a 30-minute conversation, states the stage honestly, and names the compensation range from `/docs/02_GROUP_STRUCTURE.md` \$4.2 (PROPOSED).
4. First two UK discovery interviews booked for Days 24–25.
5. Sales quota; proposal follow-ups; the best-progressed proposal is moved to contract discussion with a target signature by Day 30.

Why — Master-audit problem 6 (unvalidated pricing) must be closed before any Nigeria conversation; the Ekiti call and the adviser approaches are inputs to gate G-D and to product credibility respectively.

Deliverable — Decision D-006; repository and website free of the £300 figure as a price; Ekiti call request sent; three approach letters; two interviews booked.

Expected result — Every number Vytalix now quotes can be defended from cost-to-serve; the Nigeria story starts moving from "unknown" to KNOWN.

Day 23 — Tuesday 6 October 2026 · Article A1 published; deck v1; Post 10

Action

1. Article A1 ("DTAC without the panic: a founder's map of NHS digital assurance") published as a LinkedIn article today; mirrored on the website Insights page at launch on Day 26.
2. Publish Post 10 ("The £300-per-woman question: how to price a maternal health programme honestly") — consistent with the memo adopted yesterday; the post states the figure was "proposed" and is being re-based, nothing more.

3. Investor deck v1 (stage-honest) drafting for the data room: what exists (VERIFIED), what is in development, the services business, the round and use of funds (PROPOSED), risks.
4. SEIS/EIS draft reviewed with the accountant; outstanding items listed.
5. Discovery calls; 8 new contacts; 5 follow-ups.

Deliverable — A1 live; Post 10; deck v1 draft; SEIS/EIS review note.

Expected result — A1 begins to earn K1-cluster search traffic (long-term; no target inside 30 days); the deck exists for solicitor review in Week 5.

Day 24 — Wednesday 7 October 2026 · Partial Month 0 close; UK interviews 1–2; audit draft; Post 11

Action

1. Partial Month 0 close with the accountant (founder OS \$2.3, Wed 7 Oct): bank reconciliation (account only days old), spend to date versus the \$5 budget, cash and runway line. The audit re-score is Day 31 (task 2.22).
2. UK discovery interviews 1 and 2 held (30 minutes, consented, no patient data); notes coded for personas and Report 1.
3. Prototype audit draft report received from the fractional CTO; findings triaged (pass / conditions / fail) for the Day 30 final.
4. Advisory-board approaches 4–6 sent. Publish Post 11 ("Week 4: first proposals out. What buyers pushed back on." — real objections only; if none yet, the post says so).

Deliverable — Month 0 close note; two interview records; audit draft; three more approach letters; Post 11.

Expected result — Vytalix has its first home-market evidence and a first view of whether the prototype is reusable.

Day 25 — Thursday 8 October 2026 · Newsletter issue 1; event 1; first engagement negotiation; Post 12

Action

1. Newsletter issue 1 sent Thursday 08:00 (format per `/docs/14_MARKETING_AND_CONTENT_ENGINE.md` §3.6) if the list has been built lawfully (double opt-in; ICO registration in place). If the list is too small to be worth an issue, hold it to Fri 23 Oct (task 3.17) and log the reason.
2. Event 1 attended if booked (dates TO VALIDATE); same-day CRM update; follow-ups within 48 hours.
3. First engagement: contract discussion with the leading prospect; SOW drafted on the solicitor's template (returned tomorrow) or, if signature must precede that, on the personal-contract-plus-novation route the solicitor confirmed on Day 9; deposit invoice prepared.
4. Publish Post 12 ("DTAC readiness self-check: 20 items", carousel).
5. UK interviews 3–4 booked for Days 29–30.

Deliverable — Issue 1 sent or deferred with reason; event notes in CRM; SOW draft; Post 12; two interviews booked.

Expected result — The first engagement is at "verbal yes, contract in review" by tonight (PROPOSED), with signature on Day 29–30.

Day 26 — Friday 9 October 2026 · Website live; Ekiti status call; data room v1; Friday review 4

Action

1. Website v1 published from `/website` with privacy, cookie and terms pages, entity details, ICO reference, newsletter sign-up with double opt-in, Insights page carrying A1 (task 2.12). Search Console confirmed; sitemap submitted.
2. Ekiti status call held (task 2.14): facts only — did the ministry reply after March 2026; is there a budget line; who owns the relationship; what does the state say about its existing programme (mDoc's Digital Mom Project, VERIFIED via public reporting). Logged as KNOWN. No offer made. This is an input to gate G-D on Day 60.
3. Contract templates returned by the solicitor (task 2.7); MSA/SOW/NDA adopted.
4. Advisory board: six approaches sent, three conversations booked (task 2.15 KPI).
5. Data room v1 index complete and one-pager v1 finished (task 2.16); SEIS/EIS application drafted (2.17; submission Day 30); grant calendar complete with at least one eligible call (2.20); bid/no-bid to be scored with `/docs/20_DECISION_FRAMEWORK.md` on Fri 23 Oct (task 3.16).
6. Content batch: posts 13–15; Friday review 4; pipeline clean; KPI update; cash check; time audit.

Why — This is the last weekday before the Day 30 exit; five parent-plan tasks land here.

Deliverable — Site live; Ekiti call note; adopted templates; three adviser conversations booked; data room index and one-pager v1; SEIS/EIS draft; grant calendar; Friday review 4.

Expected result — Pre-condition 4 is met; the Nigeria question has facts; the company has a conversion surface for everything published so far.

Day 27 — Saturday 10 October 2026 · Light day

Action — Read the prototype audit draft in full; note questions for the CTO; assemble the board pack (agenda, policies to adopt, decisions D-001 to D-006 for ratification, pre-condition tracker, cash line). ≤ 2 hours.

Deliverable — Audit questions; board pack draft.

Expected result — Tuesday's board meeting is a 60-minute decision session, not a briefing.

Day 28 — Sunday 11 October 2026 · Light day; Day 30 checklist

Action — Week 5 plan; Day 30 readiness checklist (first SOW; deed or licence; audit final; SEIS/EIS submission; board meeting; four interviews); board agenda circulated to the solicitor and accountant for attendance or written input.

Deliverable — Week 5 plan; checklist with owner and time for each Day 30 item.

Expected result — Every Day 30 item has a named hour on Monday or Tuesday.

Day 29 — Monday 12 October 2026 · Signing day preparation; investor pack re-verification

Action

1. IP deed (or licence) execution scheduled with the solicitor and David Agunede for Day 30 (task 2.11); execution version circulated; cap-table entry prepared; if the document is not ready, the Day 30 slip is logged with a new date no later than Day 60 (hard stop).
2. First engagement: SOW signature pushed for today or tomorrow; deposit invoice ready to raise on signature.
3. UK discovery interview 3 held.
4. Investor pack: re-verify the ten names' team pages, mandates and routes (preview of task 3.2); "Last Verified" updated in `/investors/VYTALIX_INVESTOR_CRM.csv`; one personalised reason line written per investor (a named portfolio company, a public thesis statement, or a public statement by the partner). Still no outreach.
5. SEIS/EIS application final check with the accountant for tomorrow's submission.

Deliverable — Signing appointment; SOW at signature stage; interview record 3; investor pack v2 with re-verified routes and personalised lines; SEIS/EIS final.

Expected result — Day 30 is execution only.

Day 30 — Tuesday 13 October 2026 · Phase 2 exit

Action

1. First signed engagement (task 2.8): SOW countersigned; first invoice raised the same day (£3k–£10k, ESTIMATE); kick-off booked within three days (SOP 06).
2. IP deed executed with David Agunede (gate G-B), or the exclusive licence documented, or the slip logged against the Day 60 hard stop (task 2.11). Cap table updated; "Maternal Link" signature blocks and any parallel materials retired under the agreed terms.
3. Prototype audit final report received (task 2.10): pass / conditions / fail, with the reuse-versus-rebuild recommendation feeding the MVP decision on Day 86.
4. SEIS/EIS advance-assurance application submitted (task 2.17) — from now the word is "applied for".
5. First board meeting (task 2.18): policies adopted (conflicts, anti-bribery, data protection, clinical-claims sign-off, delegated authorities); SOPs adopted; decisions D-001 to D-006 ratified; G-B outcome minuted; independent NED search opened; pre-condition status for Day 31 recorded.
6. UK discovery interview 4 held (task 2.19 KPI: four by Day 30).
7. Publish Post 13 ("Ekiti already has a digital maternal programme. That's good news, if you design for it.", Nigeria Health Watch cited).
8. Log the Day 31 pre-condition check outcome: if all five are met, the first ten investor calls begin Wed 14 Oct with the pack from Day 29 and solicitor sign-off on the deck disclaimers (task 3.1); if any is open, investor tasks wait and market-validation tasks proceed.

Why — The parent plan's Phase 2 exit criteria: incorporated; IP deed signed; first SOW signed; prototype audit report; data room v1; SEIS/EIS submitted; first board meeting held.

Deliverable — Signed SOW and invoice; signed deed or licence (or dated slip); audit report; HMRC submission receipt; board minutes; four interview records; Post 13; pre-condition log.

Expected result — Vytalix is a trading company that owns or licenses what it says it is building, with revenue on the books, an audited prototype, an honest data room and a lawful basis to open investor conversations the following morning.

4. What "done" looks like on Day 30 (PROPOSED scorecard)

Dimension	Day 1 (VERIFIED)	Day 30 target	Parent-plan task
Entity	None	Incorporated (Day 10–12); bank; Xero; ICO; PI/PL insurance; contract templates adopted	2.1–2.4, 2.7
Name	Two spellings; four collisions	Cleared or reserve name adopted; domain and handles held; "Vitalyx" retired; trademark filing instructed for Day 45	1.2, 1.11
IP	Undocumented	Standstill signed (Day 5); heads of terms sent (Day 7); deed executed or licence documented (Day 30); founder IP assigned (Day 10); prototype audited	1.4–1.6, 2.10, 2.11
Claims	2025 deck in circulation	Intended-use statement adopted (Day 3); zero external artefacts with retired claims	1.7
Revenue	£0	40 touches Week 1; full quota from Week 3; 5 discovery calls and 2 proposals by Day 14; first signed engagement and invoice Day 30	1.10, 2.5, 2.6, 2.8
Pricing	£300/woman/year in a proposal	Re-base memo adopted Day 22; £6–£40 platform layer (ESTIMATE) in all materials	2.9
Investors	319 researched, 0 contacted	Pack for ten calls prepared and re-verified; data room v1; one-pager v1; SEIS/EIS applied for; zero outreach	1.8, 2.16, 2.17
Market validation	0 UK conversations; Ekiti unknown	30-name interview list; 4 UK interviews; Ekiti status logged as KNOWN	2.14, 2.19
Credibility	None	Company page; website live; 13 posts; Article A1; newsletter issue 1 (or deferred with reason); 6 adviser approaches, 3 conversations	2.12, 2.13, 2.15
Governance	None	First board meeting; policies and SOPs adopted; decision log D-001–D-006	2.18
Audit score	11	≥ 20 at the Day 31 re-score (target)	2.22

5. Thirty-day cash budget (ESTIMATE; subset of /docs/18_90_DAY_EXECUTION_PLAN.md §8)

Item	Day	Low (£)	High (£)
Solicitor — first tranche (standstill, heads of terms, founder assignment, deed or licence, templates, name opinion)	4–30	3,000	7,000
Trade-mark attorney opinion (filing itself is Day 45)	4	300	600
Incorporation fee, registered office, ICO Tier 1 (fees TO VALIDATE)	10–15	150	350
Accountant (first month) and Xero	5–30	100	250

Item	Day	Low (£)	High (£)
Insurance (PI + PL; first instalment or annual)	16	800	2,000
Domain(s), Workspace, CRM (free tier), newsletter tool (free tier), design tool	5–19	100	300
Prototype audit (fractional CTO, fixed fee)	10–30	1,000	2,500
Event 1 and travel	25	50	150
Contingency	—	1,000	2,000
Total		6,500	15,150

Deferral order if founder capital (TO CONFIRM) is short: contingency → event → design tool → prototype audit reduced to a code-provenance and data check only (the reuse decision then waits) → insurance bound at the first engagement date rather than Day 16. The solicitor tranche and the incorporation costs are not deferrable.

Priorities / Risks / Next actions

Priorities

- Days 1–7: the four blockers — name cleared, incorporation decided, David Agunede conversation held with standstill signed and heads of terms sent, claims retired. Nothing else in the 90 days is possible without them.
- Revenue every weekday from Day 8; full quota from Day 15; first signed engagement and invoice on Day 30.
- Investor calls are prepared, re-verified and held back until Day 31; the pack exists so that not one day is lost when the pre-conditions close.
- Facts before offers on Nigeria: pricing re-based (Day 22) and the Ekiti status learned (Day 26) before anything is proposed to anyone.

Risks

Risk	Likelihood	Impact	Mitigation in this plan
David Agunede does not sign the standstill or respond to the heads of terms	Medium	High	Standstill offered Day 3, chased Day 5; response date Day 19; licence fallback drafted in parallel from Day 17 if signals are negative; Day 60 hard stop and clean-room rebuild stand
Name clearance forces a rename or a reserve-name incorporation	Medium	Medium	Decision rule fixed on Day 1; reserve name ready; no paid brand assets before Day 5; Post 8 tells the story honestly either way
Incorporation, bank or ICO slower than assumed	Medium	Medium	Filing on Day 10 with two working days' buffer; parallel bank applications from Day 15; personal-contract-plus-novation route for an early engagement
Sales quota missed while legal work consumes Weeks 1–3	High	High	Revenue block is always first; legal batched into 11:15 block; Day 14 remedial check; VA trigger monitored from Day 15
Prototype audit blocked by lack of access or finds real patient data / third-party code	Medium	High	Access requested under the standstill on Day 8; any real-data finding is a stop-and-report event to the solicitor; reuse-versus-rebuild decided on evidence, not hope

Risk	Likelihood	Impact	Mitigation in this plan
Temptation to start investor outreach before the pre-conditions	Medium	High	Pack is marked "not for use"; pre-condition check is a logged board decision on Day 30/31; solicitor sign-off required on deck wording
Founder capital below the P0 bench	Medium	High	\$5 deferral order; first engagement deposit targeted for Day 30
Kemek Enterprise demands exceed 30% of the week	Medium	High	Time audit line in every Friday review; conflicts register; board visibility from Day 30

Next actions

Action	Owner	Deadline
Run name searches (steps 1–4), log G-A decision, send David Agunede meeting request and questionnaire, open investor call pack	Founder	Mon 14 Sept 2026
Hold the David Agunede conversation; adopt the intended-use statement; retire the 2025 claims	Founder	Wed 16 Sept 2026
Name decision; standstill signed; solicitor and accountant instructed; 40 touches sent; investor pack v1	Founder	Fri 18 Sept 2026
Send IP heads of terms	Founder + solicitor	Sun 20 Sept 2026
File incorporation; sign founder IP assignment	Founder + accountant + solicitor	Wed 23 Sept 2026 (hard Fri 25 Sept)
Bank, ICO, insurance, newsletter tool, interview list; heads-of-terms response logged	Founder	Fri 2 Oct 2026
Pricing memo adopted; website live; Ekiti status call; data room v1	Founder	Mon 5 – Fri 9 Oct 2026
First SOW signed; IP deed executed; audit report; SEIS/EIS submitted; first board meeting; pre-condition log	Founder; board	Tue 13 Oct 2026
Pre-condition check; first ten investor calls begin only if all five are met	Founder + solicitor	Wed 14 Oct 2026